

Master of Public Health (MPH) Applied Practice Experience

Wayne State University School of Medicine
Department of Family Medicine and Public Health Sciences
Master of Public Health Program

January 2025 – June 2025

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Welcome,

This portfolio includes some of the abstracts and posters completed by Wayne State University Master of Public Health (MPH) students in FPH 7440: Applied Practice Experience between January 2025 and June 2025.

We want to extend our gratitude to our incredible community partners and preceptors for their support and mentorship.

Name: Al Musawa, Mohammed

Site: Michigan Antibiotic Resistance Reduction (MARR) Coalition

Title: Supporting Marketing and Transition of Adult Educational Programs to Ferris State University

Introduction: The Michigan Antibiotic Resistance Reduction (MARR) Coalition, a nonprofit focused on combating antibiotic resistance in Michigan through community education, is undergoing a significant transition. Management of MARR is being transferred to Ferris State University (FSU), a move that will shape its future. This practicum supports the transition by formalizing educational protocols, enhancing program materials, and ensuring a seamless transfer of resources. These efforts aim to sustain MARR's public health impact.

Methods: Two primary objectives guided this practicum: (1) to document current policies and procedures for the MARR Antibiotic Ambassadors' adult education programs into a manual, and (2) to facilitate the transfer of the MARR website to FSU. This involved aligning educational content, branding, and functionality with FSU standards and helping create a dashboard for MARR activities. Key activities included delivering two "Antibiotics and You" presentations at community centers, updating adult-focused slide content, and drafting a recruitment video transcript to attract new Ambassadors.

Outcomes: Three significant outcomes emerged from this practicum, all the result of collaborative efforts. The successful formalization of Ambassador policies, creation of a recruitment video transcript, and updating of "Antibiotics and You" slides to align with FSU standards were all achieved through teamwork. This practicum enhanced my skills in public health program design, stakeholder communication, and evidence-based interventions, while demonstrating the power of collaboration. Community presentations also broadened outreach.



Supporting Marketing and Transition of Adult Educational Programs to Ferris State University

Mohammed Al Musawa, PharmD, BCIDP, Master of Public Health Candidate
Elaine M. Bailey, PharmD, Executive Director
Michigan Antibiotic Resistance Reduction (MARR) Coalition

Introduction

The Michigan Antibiotic Resistance Reduction (MARR) Coalition, a nonprofit dedicated to combating antibiotic resistance in Michigan through community education, is undergoing a significant transition. The management of MARR is being transferred to Ferris State University (FSU), a crucial step that will shape the organization's future. This practicum, therefore, is designed to support this transition by formalizing educational protocols, enhancing program materials, and ensuring a seamless transfer of resources. These efforts are aimed at sustaining the public health impact that MARR has been making.

Deliverables Grid

Public Health Competency	Deliverable 1 - Two-Minute Video Outline: Inspire Healthcare Students to Become MARR Ambassadors	Deliverable 2 - MARR Educational Programming: Current State and Future Goals
Design a population-based policy, program, project, or intervention. (Foundational-9)		X
Propose strategies to identify stakeholders and build coalitions and partnerships for influencing public health outcomes. (Foundational-13)	X	
Communicate audience-appropriate (i.e., non-academic, non-peer audience) public health content, both in writing and through oral presentation. (Foundational-19)	X	X
Adapt an evidence-based intervention to address a public health problem in an urban population with community partners. (Concentration-3)		X
Appraise public health data disseminated through health communication messaging. (Concentration-4)	X	X

Methods

Objective 1

To document current policies and procedures for the MARR Antibiotic Ambassadors' adult educational programs into a manual

Objective 2

To facilitate the transfer of the MARR website to FSU. This involved aligning educational content, branding, and functionality with FSU's standards and helping to create a dashboard for MARR activities

Key activities included delivering two "Antibiotics and You" presentations at community centers, updating adult-focused slide content, and drafting a recruitment video transcript for attracting new Ambassadors.

Results

Outcome 1

Successfully formulated the ambassador policy

Outcome 2

Created a transcript for the recruitment video

Outcome 3

Updated the 'antibiotics and you' slides to align with FSU standards

Future Directions

MARR plans to expand educational programs to youth and healthcare providers while enhancing digital outreach with online resources. Ongoing evaluations will ensure effectiveness, and partnerships with local organizations will strengthen community engagement. The goal is to maintain a sustainable framework for public health impact.

Conclusion

This practicum not only enhanced my abilities in public health program design, stakeholder communication, and evidence-based interventions, but also demonstrated the power of collaboration in achieving our goals. Additionally, community presentations broadened outreach.

Name: Alshumari, Duaa

Site: Henry Ford Health - Department of Family Medicine

Title: Advancing Reproductive Health Equity and Community Engagement

Introduction: This practicum focuses on supporting Henry Ford Health's initiatives to advance reproductive health equity through data analysis, community outreach, and collaborative efforts. The Department of Family Medicine is dedicated to improving reproductive health, addressing maternal mortality, and promoting breastfeeding, especially within marginalized communities. The primary goal is to enhance existing programs that address disparities in reproductive health outcomes, focusing on health justice and equity.

Methods: The practicum will involve a combination of data analysis, community engagement, and clinical observation. Data analysis will center on reviewing and updating existing reproductive health curricula to ensure that they reflect the latest evidence and address current community needs. The data will be analyzed to assess program impacts and make recommendations for improvement. Outreach events will be planned and coordinated with healthcare professionals, and I will assist in creating a community health resource directory. Additionally, I will shadow healthcare providers to gain first-hand understanding of maternal care and breastfeeding practices, ensuring that these interventions are culturally sensitive and evidence-based.

Outcomes: The expected outcomes include a comprehensive report and poster analyzing the impact of reproductive health initiatives, providing evidence-based recommendations for future practices. A digital community health resource guide will also be created, offering vital information on services supporting reproductive justice, maternal health, and breastfeeding. This project will contribute to advancing public health practices and improving reproductive health outcomes by fostering community engagement and utilizing data to inform strategies for health equity.



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Advancing Reproductive Health Equity And Community Engagement

Duaa Alshumari, Master of Public Health Candidate

Denise-White Perkins, MD, PhD, Chair, Department of Family Medicine and Director, Healthcare Equity Initiative,
Office of System Diversity Equity and Inclusion Henry Ford Health
Kelita Fox, MD, Family Medicine Physician Henry Ford Health

Introduction

- This practicum project focuses on supporting Henry Ford Health's initiatives to advance reproductive health equity through data analysis, community outreach, and collaborative efforts with the Department of Family Medicine. The primary aim is to improve reproductive health outcomes by addressing disparities, particularly within marginalized communities. The Department of Family Medicine is dedicated to improving reproductive health, reducing maternal mortality, and promoting breastfeeding, with a strong focus on health justice and equity. These efforts are essential to advancing reproductive health justice and ensuring that all individuals, regardless of background, receive high-quality, culturally sensitive care.
- The practicum addresses these goals by analyzing existing reproductive health programs, assessing their impact, and providing recommendations for improvement. In addition, the project will refine and enhance the department's curriculum and outreach initiatives. By engaging with healthcare professionals and the community, the practicum will promote more inclusive healthcare services, while integrating trauma-informed care principles to ensure that interventions are sensitive to the needs of marginalized populations. Ultimately, this project will contribute to reducing health inequities and improving reproductive health outcomes through data-driven insights and collaborative strategies.

Methods

This practicum will use a combination of curriculum review, community engagement, and clinical observation to evaluate and enhance reproductive health initiatives. The approach will focus on assessing existing programs, refining the reproductive health curriculum, and fostering community partnerships.

Curriculum Review and Enhancement: Evaluate and update the reproductive health curriculum by researching current best practices and integrating new, relevant information to improve the program's effectiveness.

Community Engagement: Collaborate with healthcare professionals to plan and coordinate outreach events, raising awareness of reproductive health resources in marginalized communities.

Clinical Observation: Shadow healthcare providers in clinical settings to observe maternal care and breastfeeding practices, with a focus on integrating trauma-informed care.

Creation of Health Resource Guide: Develop a digital community health resource directory to provide accessible information on reproductive health services offered by Henry Ford Health's Department of Family Medicine, as well as resources on various women's health and reproductive topics.

Deliverables



Figure 1: Women's Health and Reproductive Care Resource Library

A digital guide providing information on reproductive health services, maternal health, and breastfeeding, with an emphasis on supporting marginalized communities.

2024 Analysis Report on the Recommendations for Henry Ford Health System (HFHS)
Department of Family Medicine Curriculum on Reproductive Justice and Maternal Health

This document serves as an amplification of the 2024 Recommendations for HFHS Department of Family Medicine Curriculum on Reproductive Justice and Maternal Health, providing up-to-date analysis, new insights, and contextual understanding of key topics. This report not only highlights the importance of new developments in maternal health but also explains how integrating trauma-informed care, analyzing current resources, and providing additional relevant information will improve outcomes for mothers, families, and communities.

The amplified report includes the following key components:

1. Incorporation of Trauma-Informed Care and Its Significance
2. Evaluation of Current Local and National Resources
3. Addition of New Data, Models, and Initiatives
4. Analysis of Gaps in Maternal and Infant Health Care

1. Incorporating Trauma-Informed Care

Trauma-Informed Care (TIC) is a framework that understands, recognizes, and responds to the effects of trauma. It is a strength-based approach that is rooted in recognizing and addressing the effects of trauma. It prioritizes physical, psychological, and emotional safety for both providers and survivors, while also creating opportunities for survivors to regain control and empowerment. This is especially crucial for birthing individuals who have experienced adverse life events, such as violence, abuse, or discrimination.

Figure 2: 2024 Analysis Report

A report evaluating and recommending updates to the HFHS Family Medicine Curriculum on Reproductive Justice and Maternal Health, focusing on trauma-informed care and addressing gaps in services.

Future Implications

Community Engagement: Continued outreach and education efforts will strengthen connections with marginalized communities, ensuring they remain informed and empowered to access reproductive health services.

Expanding Access to Care: Future efforts could focus on broadening the distribution of the resource guide, making it more widely available and ensuring that underserved populations can easily access reproductive health services.

Sustainability and Growth: The project could evolve into a larger-scale initiative, with ongoing updates to the resource guide, creating an ever-evolving support system for individuals seeking reproductive health information and care.

Curriculum Enhancement: Recommendations for the HFHS Department of Family Medicine curriculum on reproductive justice and maternal health will continue to be updated, incorporating new evidence, practices, and improvements to ensure the curriculum remains relevant and effective.

HENRY FORD HEALTH

Conclusion

By integrating trauma-informed care and enhancing community outreach, it aims to improve maternal health outcomes. The deliverables, including the analysis report and resource guide, provide essential tools to support efforts in reducing maternal mortality and improving access to reproductive healthcare.

As healthcare systems prioritize equity, the insights and resources from this practicum will help guide future practices, ensuring services are culturally sensitive and inclusive. Ongoing community engagement and trauma-informed care will be vital for sustaining improvements in reproductive health.

References

1. A Closer Look At The Mental Health Provider Shortage. (n.d.). www.counseling.org/publications/counselingtoday/magazine/article-archive/article-encyclopedia-look-at-the-mental-health-provider-shortage
2. About Us. (2013, March 28). The Women-Inspired Neighborhood (WIN) Network. <https://www.winnetworkdetroit.org/about-us/>
3. FAQ. (n.d.). Birth Detroit. <https://www.birtheatdetroit.com/faq>
4. Hooper, E. K., Bassuk, E. L., & Olivet, J. (2022). Shelter from the Storm: Trauma-Informed Care in Homeless Services Settings. *The Open Health Services and Policy Journal*, 3(1). Article 1. <https://doi.org/10.2174/1874924001003010080>
5. How the U.S. Is Failing Moms on Maternal Mental Health Care | Psychology Today. (n.d.). <https://www.psychologytoday.com/us/blog/the-fainting-couch/202308/how-the-united-states-is-failing-moms-on-maternal-mental-health-care>
6. About Us. (2022, May 15). Coalition on Temporary Shelter. <https://coatsdetroit.org/about-us/>
7. Lu, M. C., Kozlowski, M., Hogan, V., Jones, L., Wright, K., & Hallon, N. (2010). Closing The Racial-White Gap In Birth Outcomes: A Life-Course Approach. *Ethnicity & Disease*, 20(1, Suppl 2), S2-62-76.
8. Maternal Health. (n.d.). The Southeast Michigan Perinatal Quality Improvement Coalition. <https://www.sempic.org/maternal-health>
9. Our Work. (n.d.). Hope Starts Here. <https://hopesstartshere.org/our-work/>
10. Patient Screening. (n.d.). The American College of Obstetricians and Gynecologists. <https://www.acog.org/programs/perinatal-mental-health/patient-screening>
11. Restoring Access to Maternity Care in Rural America | Commonwealth Fund. (n.d.). <https://www.commonwealthfund.org/publications/2021/sep/restoring-access-maternity-care-rural-america>
12. Setting New Standards with Trauma-Informed Care. (n.d.). Pennsylvania Psychiatric Institute. <https://ppimhs.org/newsroom/setting-new-standards-with-trauma-informed-care/>
13. U.S. Department of Health and Human Services. Office on Women's Health. (n.d.). <https://www.womenshealth.gov/>

Name: Al-Souri, Dunya

Site: Henry Ford Health - Behavioral Health Services

Title: Improving Behavioral Health Access: A Data-Driven Approach

Introduction: Henry Ford Behavioral Health in Detroit is committed to delivering personalized mental health care. Access to timely behavioral health services remains a challenge, contributing to delays in care. In December 2024, the organization implemented a new triaging tool and referral management process. This practicum aimed to evaluate the tool's impact on patient access and identify areas for improvement.

Methods: I conducted a mixed-methods evaluation, including quantitative comparisons of baseline data to post-implementation triage data, along with quantitative and open-ended feedback from 26 behavioral providers. Using Microsoft Forms, I surveyed providers on the tool's effectiveness, usability, and integration into clinic operations. Findings were shared with Henry Ford Behavioral Health stakeholders through a PowerPoint presentation summarizing referral and scheduling trends, provider insights, and recommendations.

Outcomes: The improved tool's rubric reduced the average wait time from referral to scheduled appointment by 5 days. Survey responses showed that 50% of providers found the tool beneficial in scheduling the right patients at the right time. 73% of providers reported that more than a quarter of their crisis patients presented with a higher level of severity than were categorized on their referral, suggesting refinement to the rubric. I recommend a follow-up evaluation in June 2025 to assess the impact of rubric updates made in March 2025. Ongoing evaluation will ensure the tool continues to improve behavioral health access.



Improving Behavioral Health Access: A Data-Driven Approach

Dunya Al-Souri, BSPH, Master of Public Health Candidate
Shamole Ahmed, MPH, CLSSBB
Henry Ford Health – Behavioral Health Services (Outpatient)

Introduction

- Henry Ford Behavioral Health (HFBH) in Detroit aims to provide timely and personalized mental health care.
- Providers have been facing delays in scheduling due to limited triage information, resulting in patients being scheduled based on referral date rather than clinical urgency.
- To address this, HFBH implemented a triage tool and referral management process in December 2024.
- The tool categorizes referrals from both non-behavioral health providers (ambulatory) and behavioral health providers (treatment order) using severity levels: emergent, urgent, and routine.
- In March 2025, an "imperative" level that comes after urgent was added to improve prioritization.
- This practicum evaluated the tool's impact on access and identified areas for improvement.

Methods

Survey Development and Feedback Collection

- Created a Microsoft Forms survey to collect both quantitative and open-ended provider feedback on the tool's usability and impact.

Referral Data Tracking

- Generated weekly Excel triaging reports from February 2025 to March 2025 using pivot tables to track referral priority trends.
- Created a master Excel dataset combining triage data from December 2024 to March 2025.

Operational Metrics Tracking

- Counted weekly unfilled provider authorized only (PAO) scheduling slots.
- Counted high no-show and cancellation rates by provider.

Data Analysis and Visualization

- Identified comparison metrics before and after tool implementation.
- Designed visual data outputs to highlight trends in referral priority, scheduling delays, and provider feedback.
- Analyzed findings to understand the tool's strengths and gaps.

Dissemination

- Presented findings to HFBH stakeholders through a PowerPoint summarizing access improvements, ongoing challenges, and provider feedback.

Deliverables

Two deliverables were created during the practicum:

- A PowerPoint was presented to HFBH stakeholders highlighting key metrics, provider insights, and recommendations.
- A Microsoft Form survey developed to gather provider feedback on the triaging tool's impact on access.

Average Number of Lead Days
from Referral to Scheduled Appointment

Priority Level	Order Type	Baseline (Jan. 2024 - Oct. 2024)	Triage (Dec. 2024 - Feb. 2025)	Triage Updated Rubric (Mar. 2025)
Emergent	Ambulatory	16	16	12 (+4)
Emergent	Treatment Order	8	46 (↑38)	39 (↓7)
Urgent	Ambulatory	19	22 (↑3)	16 (↓6)
Urgent	Treatment Order	39	49 (↑10)	45 (↓4)

Figure 1: Quantitative data showing average lead days from receiving a referral to scheduled appointment increased after initial triage implementation (red) but decreased after the rubric update (green).

Percentage of Crisis Patients Having
Presented with Higher Acuity or
Severity

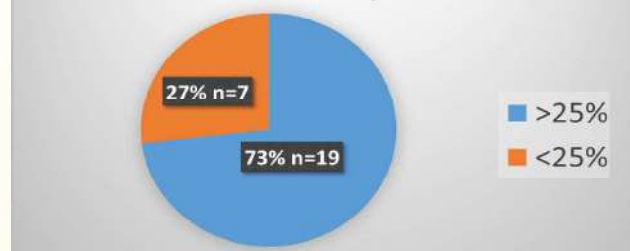


Figure 2: Quantitative survey data of provider reports on crisis patients presenting with higher mental health needs than indicated by the tool.

Results

Changes in Referral and Scheduling Patterns

- The tool and updated rubric improved referral accuracy by increasing urgent and emergent referrals while reducing routine ones, helping ensure patients are prioritized based on need.
- Average wait times from referral to appointment initially rose after the tool's launch, but decreased following the rubric update, which indicated improved access to care (see Figure 1).

Quantitative Provider Survey Feedback

- 50% of providers felt the tool improved scheduling and patient prioritization, while the other half found it neutral or not beneficial.
- 73% of providers reported that over 25% of their patients had greater mental health needs than indicated by the tool (see Figure 2).

Qualitative Provider Survey Feedback

- Provider feedback was generally positive, highlighting improved appointment flow and response times, though concerns were raised about underestimating patient mental health needs and prioritization.
- Provider suggestions for the tool's improvement included better education on scheduling and more training for primary care physicians to accurately assess patient urgency.

Conclusion

- The triaging tool and updated rubric positively impacted behavioral health access by improving referral accuracy and reducing wait times, though further adjustments are needed to better prioritize patient needs.
- Follow-up evaluation in June 2025 is recommended to assess whether rubric changes continue to improve access, as well as identify ongoing areas for improvement.
- For leadership, I recommend improving PAO slot utilization to ensure all slots on a schedule are filled, as well as addressing high no-show and cancellation rates to improve access.
- Ongoing evaluation, provider training, and strong communication between teams are key to sustaining improvements to behavioral health access.

Name: Bernardo, M.

Site: Wayne State University Mobile Health Unit

Title: Designing a Wayne Mobile Health Unit Evaluation Manual

Introduction: The Wayne Mobile Health Unit (WMHU) program aims to facilitate access to preventative healthcare in the Metropolitan Detroit Area. With the constant expansion of the WMHU, it is important that the program is evaluated annually to determine any needs for improvement. The purpose of this project is to develop an evaluation manual for the WMHU Operations team evaluation procedures to ensure the program runs effectively and efficiently.

Methods: To develop the evaluation manual, we began with a literature review to identify best practices for programs like the WMHU. We also collected prior evaluation tools from other Wayne State University programs and talked with those that used those tools to identify what was successful and any challenges using those tools. A discussion with the current WMHU Operations team took place to seek out further data points of interest. Additionally, site visits were made to WMHU events to observe possible data points that should be collected as part of the evaluation related to the on-site workflow specifically. The completed evaluation is set to be received by the WSU Institutional Review Board to determine if it is a quality improvement project.

Outcomes: The completed manual contains detailed, focused and easy-to-follow qualitative and quantitative methodologies to evaluate: 1) participant satisfaction and feedback; 1) participant-staff communication; 2) staff self-assessment and feedback; 3) administrative staff leadership; 4) program efficiency in terms of workflow, capacity and outreach; 5) program effectiveness in terms of improved health outcomes; and, 6) accurate and comprehensive data collection and analysis.



Introduction

Healthcare in the United States (US) is characterized by:

- Increasing costs
- Decreasing access to preventive healthcare particularly among
 - Racial and ethnic minorities
 - Rural populations
 - Underprivileged and/or disenfranchised groups.¹

The WMHU program was established in 2020 to bring :

- SARS-CoV-2 virus testing and vaccination to
 - Vulnerable populations in Detroit²
- More recent WMHU offers screenings for :
 - Cardiovascular diseases
 - Diabetes
 - Human Immunodeficiency Virus (HIV)
- In partnership with Wayne Health, WMHU provides referrals to:
 - Primary and specialized physicians
 - Navigation services to community resources

WMHU aims to:

- Bring healthcare to vulnerable patients in their own neighborhoods,
- Ameliorate health inequities and reduce health disparities

amongst Metropolitan Detroit area communities.

Purpose

This evaluation will lay the foundation for :

- Program improvements
- Enhanced advocacy
- Transparent communication with stakeholders
- Informed decisions about the program's future
- Enhanced participation

Methods and Analysis

Evaluation methods include:

1. Participant feedback via periodic focus groups and open and closed ended surveys to evaluate qualitatively and quantitatively overall satisfaction and to identify participants' barriers, facilitators and areas for improvement at WHMU.
2. On-site and off-site staff feedback via self-assessments, semi-structured interviews, questionnaires and open and closed ended surveys to evaluate performance, identify gaps of knowledge or communication and pinpoint areas for improvement.
3. Quantitative data collection from participants' electronic health records (EHR), service logs, cost analysis and treatment outcomes to evaluate data accuracy, completeness, adherence to protocols and health outcomes.

Results

TABLE I – WMHU On-Site Participant Survey

	Strongly Disagree	Disagree	Neutral	Agree	Strongly Agree
1. The WMHU staff are skilled and experienced.	☐	☐	☐	☐	☐
2. The WMHU staff are supportive.	☐	☐	☐	☐	☐
3. The WMHU staff always responds to my requests and concerns.	☐	☐	☐	☐	☐
4. The WMHU staff works as a team, demonstrating coordination and professionalism.	☐	☐	☐	☐	☐
5. The wait time was reasonable.	☐	☐	☐	☐	☐
6. I received clear explanations about my health condition and next steps.	☐	☐	☐	☐	☐
7. The WMHU staff respects patient privacy and confidentiality.	☐	☐	☐	☐	☐
8. The WMHU atmosphere is welcoming.	☐	☐	☐	☐	☐
9. The WMHU procedures are easy to follow.	☐	☐	☐	☐	☐
10. The WMHU staff creates a safe and trusting environment.	☐	☐	☐	☐	☐

Results

TABLE II– WMHU Staff Self-Assessment Questionnaire

	Rarely Less than 50%	Sometimes 55%	Often 65%	Usually 75%	Regularly 85%
1. I take responsibility for my own actions.	☐	☐	☐	☐	☐
2. I am aware of how my values and beliefs affect my practice.	☐	☐	☐	☐	☐
3. The WMHU staff always responds to my requests and concerns.	☐	☐	☐	☐	☐
4. I acknowledge the limits of my knowledge or ability.	☐	☐	☐	☐	☐
5. I show initiative for my own learning.	☐	☐	☐	☐	☐
6. I show willingness to participate in changes that improve client care or practice.	☐	☐	☐	☐	☐
7. I use honesty and tact when conveying information.	☐	☐	☐	☐	☐

Outcomes

This manual provides a roadmap for future program evaluations that may result in:

- Increased WMHU efficiency and effectiveness
- Increased WMHU participation
- Improved health outcomes in the Metropolitan Detroit communities served by WMHU

Citations

- 1- Malone, N. C., Williams, M. M., Smith Fawzi, M. C., Bennet, J., Hill, C., Katz, J. N., & Oriol, N. E. (2020). Mobile health clinics in the United States. *International Journal for Equity in Health*, 19, 40. 10.1186/s12939-020-1135-2- Wayne Health Mobile Unit. Retrieved Mar 18, 2025, from <https://www.waynehealthcares.org/mobile-health-unit/>

Name: Browner, Marshea

Site: Detroit Health Department - Reproductive Health

Title: Analysis of program participant feedback, engagement levels, and outcomes: A qualitative and quantitative evaluation to identify strengths, challenges, and opportunities for growth

Introduction: This study evaluates client satisfaction and system navigation in an adolescent reproductive health program (iDecide). Using qualitative interviews, surveys, and program metric reviews, it identifies barriers to access and areas for improvement. Findings will inform strategies to enhance service delivery and program effectiveness.

Methods: This evaluation employs a mixed-methods approach, including qualitative interviews, program metric reviews, and two targeted surveys to assess client satisfaction and system navigation within an adolescent reproductive health program (iDecide).

Survey Cohorts: Attended Appointment Survey and Missed Appointment Survey

The attended appointment survey aims to evaluate client satisfaction, assess service effectiveness, and determine whether the program meets participant needs. A structured survey focusing on overall experience, service accessibility, provider interactions, and areas for improvement will be used to interview participants on the program.

The missed appointment survey aims to evaluate individuals who scheduled an appointment but did not attend. A structured survey exploring reasons/barriers for non-attendance, potential service modifications, and factors influencing future engagement will be used during interviews.

Outcomes: The surveys will identify strengths, gaps, and barriers within the adolescent reproductive health program (iDecide). The findings will inform strategies to enhance satisfaction, reduce barriers, and improve service delivery.



Enhancing Service Delivery: Client Perspectives on Adolescent Reproductive Health Programs

Marshea Browner, MS, Master of Public Health Candidate
Megan Boyce, MPH- iDecide Detroit Program Manager
The Detroit Health Department- iDecide Detroit Program

Introduction

This study assesses client satisfaction and system navigation within an adolescent reproductive health program. Through surveys, qualitative interviews, and program metric reviews, we identify access barriers and opportunities for improvement. Insights from this evaluation will guide strategies to enhance service delivery and optimize program effectiveness.

Methods

This evaluation utilizes a **mixed-methods approach**, incorporating **qualitative interviews, program metric reviews, and two targeted surveys** to assess client satisfaction and system navigation within an adolescent reproductive health program.

Survey Cohorts:

•Attended Appointment Survey

- Evaluates client satisfaction and service effectiveness.
- Assesses accessibility, provider interactions, and overall experience.
- Identifies areas for improvement based on participant feedback.

•Missed Appointment Survey

- Explores barriers preventing attendance.
- Identifies potential service modifications to improve engagement.
- Examines factors influencing future participation.

Results

The surveys **identified key strengths, gaps, and barriers** within the iDecide Detroit program.

Findings highlight areas of high client satisfaction, as well as challenges related to accessibility, appointment adherence, and service navigation.

Key insights will guide targeted strategies **to enhance client satisfaction, reduce barriers to care, and improve overall service delivery.**



Figure 1- Time it took to schedule appointment



Figure 2- Preferred appointment reminder

Conclusion

Demographics:

Gender: 67% were female, 33% male

Age: 72% of clients were age 26+.

7 In-Clinic Interviews Completed

- 85 % of clients were satisfied with the services they received
- 57% of clients booked their appointment right after learning about the program
- 30% of clients discovered the program through Social Media, Referrals, or Google

11 Phone Interviews Completed

- 100% of clients rescheduled their appointment
- 27 % of clients who missed their appointment had scheduling conflicts
- 72 % of clients preferred a reminder text

Recommendations

1. Strengthen automated reminder system
2. Implement flexible scheduling and rescheduling options
3. Maintain client engagement and follow up

Appreciation

Thank you to The Detroit Health Department iDecide Program for their partnership with Wayne State University for this practicum.

Name: Burden, Brittany

Site: Detroit Health Department - Workforce Development

Title: Developing a Leadership Training Curriculum for Senior and Executive Staff at the Detroit Health Department

Introduction: The Workforce Development division within the Detroit Health Department is committed to enhancing the skills and capabilities of the public health workforce. It is responsible for designing and implementing training programs, professional development initiatives, and workforce planning to ensure staff are well-equipped to address public health challenges. The division also focuses on recruitment, retention, and career advancement to build a skilled, diverse, and motivated team.

Methods: My practicum project involved developing a comprehensive training curriculum for the Senior and Executive Leadership staff tier. After conducting a thorough analysis of survey data to identify the primary leadership needs and areas for improvement, I utilized this information to create a tailored curriculum that addressed those needs. In addition to the curriculum, I developed a detailed presentation outlining an implementation plan, which was designed for leadership to review and assess for alignment with organizational goals and objectives.

Outcomes: The outcome of my practicum project was the development of a comprehensive training curriculum for the Senior and Executive Leadership team, based on a detailed analysis of survey data that identified primary leadership needs and areas for improvement. I tailored the curriculum to address these needs, identifying targeted training opportunities to enhance skill development. Additionally, I created a presentation that delivered a review of the analysis, outlined the identified needs, and provided recommendations for an implementation plan, accompanied by the proposed curriculum. This presentation was designed to ensure alignment with organizational objectives and facilitate the integration of the curriculum into the leadership development strategy.

Developing A Leadership Training Curriculum for Senior & Executive Staff at the Detroit Health Department

Brittany Burden, Master of Public Health Candidate
Alescia Farr, Ph.D., M.P.H., Workforce Development Director
Detroit Health Department

Introduction

The Workforce Development division within the Detroit Health Department is committed to enhancing the skills and capabilities of the public health workforce. It is responsible for designing and implementing training programs, professional development initiatives, and workforce planning to ensure staff are well-equipped to address public health challenges. The division also focuses on recruitment, retention, and career advancement to build a skilled, diverse, and motivated team.

Objective: Identify training needs, resources, and appropriate training methods for each tier of City of Detroit Health Department employees.

Focus: Senior Management/Executive Level (Tier 3) competencies apply to public health professionals at a senior management level and to leaders of public health organizations.

Figure 1 - My project on the Workforce Development team was to identify the training needs and develop a training method for the Senior Management tier of staff that addressed the identified public health core competency needs.

Methods

My practicum project focused on developing a comprehensive training curriculum for the Senior and Executive Leadership team. The process began with a thorough analysis of survey data using a Competency-Based Training Framework to identify key leadership development needs and areas for improvement. To ensure the curriculum was both targeted and relevant, I employed the ADDIE Model to structure the training process, translating the survey findings into actionable goals and learning outcomes. Using this framework, I designed a tailored curriculum focused on enhancing core leadership competencies such as strategic thinking and decision-making. The curriculum was aligned with organizational goals to ensure its applicability and measurable impact. In addition to the curriculum, I developed a detailed presentation outlining an implementation plan, which was designed for leadership to review and assess its alignment with the organization's strategic objectives, ensuring that the training could be seamlessly integrated into existing frameworks.



Figure 2 - Above is the engagement and delivery plan I developed for launching the training program to staff members

Results

The outcome of my practicum project was the development of a comprehensive training curriculum for the Senior and Executive Leadership team, based on a detailed analysis of survey data that identified primary development needs and areas for improvement. I tailored the curriculum to address these needs, identifying targeted training opportunities to enhance skill development. Additionally, I created a presentation that delivered a review of the analysis, outlined the identified needs, and provided recommendations for an implementation plan, accompanied by the proposed curriculum. This presentation was designed to ensure alignment with organizational objectives and facilitate the integration of the curriculum into the workforce development strategy.

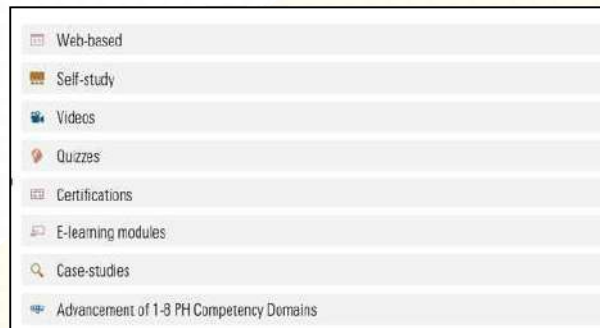
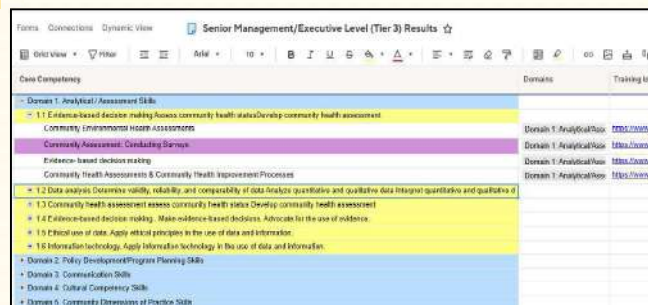


Figure 3 - This slide from my presentation to Leadership highlights the key features of the MiTrain Learning Management System. The platform includes various functionalities aimed at helping staff achieve their competencies through tailored courses, as outlined above.



Competency	Domains	Training Need
Domain 1: Analytical/Assessment Skills	Domain 1: Analytical/Assessment Skills	https://www.mitrain.com
Domain 2: Policy Development/Program Planning Skills	Domain 2: Policy Development/Program Planning Skills	https://www.mitrain.com
Domain 3: Communication Skills	Domain 3: Communication Skills	https://www.mitrain.com
Domain 4: Cultural Competency Skills	Domain 4: Cultural Competency Skills	https://www.mitrain.com
Domain 5: Community Dimensions of Practice Skills	Domain 5: Community Dimensions of Practice Skills	https://www.mitrain.com
Domain 6: Leadership Skills	Domain 6: Leadership Skills	https://www.mitrain.com
Domain 7: Systems Thinking Skills	Domain 7: Systems Thinking Skills	https://www.mitrain.com
Domain 8: Professional Development Skills	Domain 8: Professional Development Skills	https://www.mitrain.com

Figure 4 - Figure 4 - Analysis of training needs, including the identification and breakdown of key development areas.

Conclusion

Through analyzing survey data, I developed a targeted curriculum to address key development needs within the Senior and Executive Leadership team. The curriculum focused on competencies in Analytical/Assessment Skills, Policy Development, Community Dimensions of Practice, and Leadership/Systems Thinking, ensuring alignment with organizational goals. A variety of training courses will support skill development, and the curriculum also informed a workforce development plan for long-term growth. This initiative provides the leadership team with the opportunity to improve decision-making, community engagement, and systems thinking, laying the groundwork for continued development in a complex public health environment.

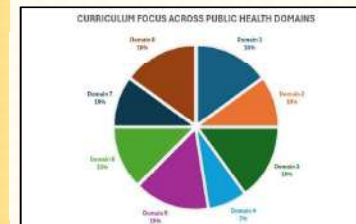


Figure 5 - This chart illustrates the focus distribution across the eight public health competency domains in the professional development curriculum, reflecting the emphasis placed on each domain based on identified personal development needs.

Recommendations

- Ongoing Evaluation and Feedback:** To ensure the curriculum remains relevant and effective, regular feedback from participants should be gathered, and course content should be updated based on emerging public health trends and team needs.
- Expand Training Opportunities:** Consider utilizing additional training platforms (e.g., online courses, workshops, webinars) to enhance accessibility and provide diverse learning experiences for all staff.
- Monitor Long-Term Impact:** Implement a follow-up plan to track the long-term outcomes of the training, focusing on improvements in leadership practices, team performance, and public health impact.
- Integrate with Organizational Strategy:** Ensure that professional development continues to be integrated with the department's broader strategic objectives, reinforcing the connection between individual growth and organizational success.

Appreciation

I would like to extend my deepest gratitude to my incredible preceptor, Dr. Alescia Farr, whose unwavering support, guidance, and mentorship were invaluable throughout this process. Dr. Farr's expert insights and feedback helped shape my work, while her direction kept me on track and helped me navigate challenges. I'm truly grateful for her generosity in sharing her time, expertise, and wisdom, and for being a steady source of encouragement every step of the way. Dr. Farr's dedication has made a lasting impact, and I'm fortunate to have had the opportunity to learn from her.

Name: Caniff, Kaylee

Site: Michigan Antibiotic Resistance Reduction (MARR) Coalition

Title: Creating Comprehensive Policies and Procedures for School-Age Programs: Facilitating the Transfer from the Michigan Antibiotic Resistance Reduction Coalition to Ferris State University

Introduction: Antibiotic resistance is a growing public health crisis affecting both individuals and communities. The MARR Coalition is a nonprofit organization dedicated to reducing unnecessary antibiotic prescribing through community-based educational initiatives. The MARR Antibiotic Ambassadors program engages health professions students to lead educational sessions in schools and community settings. Due to funding changes, MARR is currently transitioning its organizational management to Ferris State University (FSU). The aim of this practicum was to facilitate MARR's transition of school-age programs to FSU.

Methods: Key efforts included reviewing, revising, and rebranding MARR's educational materials for school-age audiences. Policies and procedures for the Antibiotic Ambassadors program were formalized into a comprehensive guide. A video script was created to support future Ambassador recruitment. Additionally, MARR's website, programming, and evaluation tools were migrated to FSU's electronic platforms, aligning them with university standards. MARR was represented at conferences and meetings to increase visibility and an educational grant proposal was authored and submitted. The transition and expansion plan was formally presented to the MARR Advisory Council.

Outcomes: The transition to FSU is progressing successfully, with major milestones achieved in content revision, policy development, and program integration. Next steps include filming a high-quality training and promotional video, securing additional funding, expanding partnerships statewide and nationally, and implementing formalized data collection to track program impact and participation.



WAYNE STATE
School of Medicine

Facilitating the Transfer of the Michigan Antibiotic Resistance Reduction Coalition to Ferris State University

Kaylee Caniff, PharmD, BCIDP, Master of Public Health Candidate
Elaine Bailey, PharmD, MARR Executive Director

Introduction

- Antibiotic resistance is a public health crisis that impacts both patients and communities.
- The Michigan Antibiotic Resistance Reduction (MARR) Coalition is a nonprofit organization focused on addressing antibiotic resistance in Michigan.
- MARR's mission is to reduce unnecessary antibiotic prescribing through community educational interventions.
- A key initiative is the MARR Antibiotic Ambassadors program, which recruits health professions students to lead educational sessions in schools and community settings.
- MARR is currently transitioning its organizational management to Ferris State University (FSU) due to loss of funding.

Practicum Aim: facilitate and support MARR's transition of school-age programs to FSU

Deliverables

1

Create a formalized policy and procedure manual for MARR Antibiotic Ambassadors

2

Present the organizational transition plan to the MARR advisory council

Methods

Key practicum tasks:

- Observed MARR presentations in elementary and middle school settings.
- Reviewed, revised and re-branded current MARR educational materials for the school-age programs.
- Formalized and documented the current policies and procedures for the MARR Antibiotic Ambassador's school-age programs into a guide.
- Created a training video script to recruit new Antibiotic Ambassadors.
- Facilitated the transfer of the MARR website, programming, and evaluations to FSU electronic platforms, ensuring that all educational content, branding, and platform functionality are fully aligned with Ferris State's standards.
- Represented MARR at meetings and conferences to increase awareness and expand reach.
- Authored and applied for an educational grant to secure funding for current and future MARR programming through the Society of Infectious Diseases Pharmacists.
- Presented the transition and expansion plan to the MARR Advisory Council.



Image 1. Observing MARR Antibiotic Ambassadors conduct a presentation in a kindergarten classroom.



Michigan Antibiotic Resistance Reduction Coalition

Conclusions

MARR's transition to FSU is progressing, with significant steps completed in content revision, policy development, and program evaluation. Integrating MARR's infrastructure within FSU provides long-term sustainability and opportunities for interdisciplinary collaboration. The continued expansion of the Antibiotic Ambassadors program will enhance public awareness to promote responsible antibiotic use, ultimately reducing antibiotic resistance across Michigan and beyond.

Recommended Next Steps

- **Film Promotional and Training Video:**
 - Partner with FSU media department to film a high-quality training and promotional video for Antibiotic Ambassador recruitment.
 - Distribute the video to current and new Antibiotic Ambassadors and advertise on social media.
- **Identifying and Securing Additional Funding:**
 - Identify potential funding sources (grants, sponsorships, donations) to sustain MARR programming long-term.
- **Identifying Partners for Expansion:**
 - Continue to reach out to potential partners (universities, community organizations, healthcare providers) to build partnerships to expand the reach of the Antibiotic Ambassadors program statewide with the goal of national expansion.
- **Tracking Metrics and Ambassador Participation:**
 - Data on the impact and reach of educational program is currently limited.
 - More formalized data collection and tracking will provide support for funding opportunities.

Name: Davis, Kevin Chadwin

Site: Michigan Clinicians for Climate Action (MiCCA)

Title: Climate Solutions are Health Solutions: Centering Human Immunodeficiency Virus (HIV) in the Climate Justice Movement

Introduction: As Dr. Lisa Del Buono reminds us, “Climate solutions are health solutions.” This practicum embodied that truth by centering environmental justice as a public health imperative. Through an intentional focus on Human Immunodeficiency Virus (HIV) care, the practicum advanced cross-sector strategies that addressed overlapping injustices—climate vulnerability, healthcare disruption, and structural inequality—while expanding coalition efforts across Michigan.

Methods: Rooted in my learning contract objectives, this practicum included leading a policy and advocacy campaign with Michigan Clinicians for Climate Action and Healthy Climate Wisconsin to oppose the proposed Paris Reciprocating Internal Combustion Engine (RICE) methane gas plant. I gathered public comments from health professionals, launched a targeted MailChimp campaign to over 20 stakeholders, and supported legislative strategy. I also attended the Our Planet, Our Health conference (March 2–4, 2025), joined workshops on climate resilience, and participated in a rally urging the United States Department of Housing and Urban Development (HUD) to recognize housing as a health determinant. As part of my education deliverable, I created and delivered a virtual webinar on March 27 titled “Climate Change and the Future of Human Immunodeficiency Virus (HIV) Prevention and Treatment,” connecting climate stressors with HIV care access.

Outcomes: This practicum demonstrated how incorporating HIV into climate justice frameworks strengthens advocacy, deepens coalition potential, and drives public health forward. Key deliverables included a nationwide webinar, a targeted resource guide developed post-presentation, policy engagement, and a forthcoming advocacy one-pager exploring the intersections of HIV care, climate change, and energy resilience in Detroit.



WAYNE STATE
School of Medicine

Climate Solutions are Health Solutions:

Centering Human Immunodeficiency Virus (HIV) in the Climate Justice Movement

Kevin Chadwin Davis, Jr
Michigan Clinicians for Climate Action, Wayne State University

Introduction

Climate change is not only an environmental crisis—it's a public health crisis, with marginalized communities facing the most severe consequences. This practicum explored how climate-related disruptions amplify existing disparities in Human Immunodeficiency Virus (HIV) prevention and care, especially among historically excluded communities. By centering equity and environmental justice, this practicum focused on developing and delivering two applied public health advocacy tools to strengthen resilience and raise awareness: (1) a professional webinar and (2) national policy engagement at the Our Planet, Our Health Conference.

Deliverables & Activities

Deliverable 1: Climate Change and the Future of Human Immunodeficiency Virus (HIV) Prevention and Treatment Webinar

This 90-minute virtual webinar was hosted on March 27, 2025, by Michigan Clinicians for Climate Action (MiCCA). It was designed for healthcare professionals, HIV prevention advocates, and public health workers. I conducted a literature review, created a 20-slide presentation, and led a statewide training session. Content covered the climate-HIV connection, Detroit's environmental risk zones, and policy strategies for more resilient HIV care systems.

Deliverable 2: Our Planet, Our Health Conference

I served as part of Michigan Clinicians for Climate Action's (MiCCA) policy delegation in Washington, D.C., attending climate and health workshops and meeting with Michigan's congressional offices. Advocacy focused on protecting funding for the Ending the HIV Epidemic (EHE) initiative, expanding clean energy access in health facilities, and supporting low-income and LGBTQIA+ communities facing climate stressors. I helped prepare district-specific materials and shared personal insights as a Detroit resident and HIV advocate.

Results

Webinar Outcomes: 30+ professionals attended; participants reported increased understanding of how climate change affects Human Immunodeficiency Virus (HIV) medication access, telehealth barriers, and systemic care gaps.

Policy Engagement Outcomes: Met with the offices of five Michigan legislators; advocated for environmental health and HIV policy integration; expanded my skills in policy storytelling, legislative briefings, and cross-sector advocacy.

Community Impact: Helped elevate Detroit's climate and HIV realities in national health equity conversations.

Figures



Figure 1. Congressional Advocacy Visit with Rep. Haley Stevens' Office

From left to right: Madison Cummin, Lauren Church, Connor Woods, Dr. Larry Junck, and Kevin Chadwin Davis meet with Representative Haley Stevens' legislative staffer, Connor Woods, to discuss the impact of the Inflation Reduction Act (IRA) on Michigan. Topics included workforce development, green energy investments, and the importance of sustaining IRA funding to protect both environmental and public health infrastructure.



Figure 2. MiCCA Delegation at the Our Planet, Our Health Conference

Members of Michigan Clinicians for Climate Action (MiCCA), including student leaders and public health advocates, gather outside George Mason University following a day of workshops on climate justice, health system decarbonization, and trauma-informed community resilience. Kevin Chadwin Davis (far right) participated as a delegate representing HIV and environmental health intersections in Detroit.



Figure 3. Learning Objectives Framing the Climate-HIV Webinar

This visual was used to open the "Climate Change and the Future of HIV Prevention and Treatment" webinar and outlined four core objectives: (1) understanding climate and HIV health disparities, (2) recognizing the localized impacts in Detroit and Michigan, (3) analyzing policy rollbacks and funding threats, and (4) mobilizing cross-sector collaboration. The session aimed to convert public health knowledge into action by connecting structural issues to actionable strategies for professionals on the ground.



Figure 4. Vivent Health & Climate Vulnerability Mapping

This slide visualized the overlap between climate vulnerability and HIV service needs in Detroit. Vivent Health, while not in the highest-risk zone, operates adjacent to ZIP codes 48208, 48211, 48213, and 48214—areas disproportionately affected by heat, infrastructure decay, and elevated HIV rates. The analysis highlighted the role of anchor health agencies in maintaining access to care during climate-induced disruptions, underscoring the need for climate-resilient infrastructure in Detroit's HIV care continuum.

Public Health Competencies

- **Foundational 6:** Addressing structural inequities and health disparities
- **Foundational 7:** Assessing population needs and capacities
- **Foundational 14:** Advocating for political and social policy improvements
- **Foundational 15:** Engaging in environmental health policy discussions
- **Concentration 2:** Proposing interventions grounded in social determinants of health
- **Concentration 3:** Synthesizing evidence with community assessment

Conclusion

This practicum affirmed my commitment to advancing climate and health equity through both education and policy advocacy. From designing and delivering a professional webinar to participating in national legislative visits, I applied key public health competencies in real-world settings that center the needs of marginalized communities. I developed tools that helped healthcare providers understand the climate-related barriers to Human Immunodeficiency Virus (HIV) care, while also advocating for systemic policy changes like multi-month prescriptions, solar-powered clinics, and the protection of federal HIV funding. Moving forward, I plan to continue building cross-sector coalitions and promoting climate-resilient HIV prevention strategies that reflect lived experience and drive structural change.

Partners



WAYNE STATE
School of Medicine



MiCCA
Michigan Clinicians for Climate Action

Name: Hankes, Angelo

Site: Michigan Department of Health and Human Services - Office of the Chief Medical Executive

Title: Learning from the Past: Improving Access to Vaccines and Highlighting Historical Communicable Disease Trends to Illustrate Vaccine Impact

Introduction: Michigan's Department of Health and Human Services (MDHHS) strives to find ways to support access to vaccines and vaccine education. However, their primary focus has been on medical or public health professionals, which require sophisticated and jargon filled explanations with visualizations. Such data becomes unwieldy when translating to distribute to the general public or clinic patient panels. Consequently, simpler visualizations are needed to facilitate vaccine education, information retention, and calls to action.

Methods: Data was gathered from the Center for Disease Control and Prevention's (CDC) Wonder platform to find mortality and morbidity rates for Influenza, COVID-19, and Measles. In the case of mortality, this specifically used searches based on the International Classification of Diseases (ICD) codes for those specific diseases and did not include related diseases that are comorbid, such as pneumonia in the case of COVID-19 or Influenza. Historical data, pre-1960, was gathered using historical national public health record for the United States from a variety of academic publishers, who have digitally preserved and archived these records. Simple plotting and graph generation were used to create the initial line and bar charts. After plotting the initial data, demarcation lines were added to make positive trends clear for increased legibility and bold colors were used to increase visibility, impact, and engagement.

Outcomes: Simple but clear graphics can be used as an educational tool for medical providers or medical science popularizers. These graphs help highlight the impact of vaccines on health outcomes and create a vaccine-positive narrative.

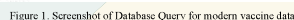


Angelo Hanks, Wayne State University

Natasha Bagdasarian, Office of the Chief Medical Executive – MDHHS and University of Michigan
Jared Welehodsky, Office of the Chief Medical Executive – MDHHS

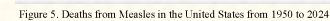
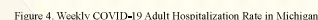
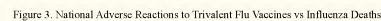
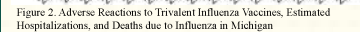
A major national concern throughout the winter was a resurgence of Measles, in combination with a general decline vaccination rates. Michigan's Department of Health and Human Services strives to find ways to support access to vaccines and vaccine education, such as dental vaccination program discussed in this presenter's companion practicum deliverables. To better inform the public on the importance of vaccines, data was gathered to create easy to understand graphs demonstrating the positive effects of vaccines and their need as a public health tool.

Data was gathered from the Center for Disease Control and Prevention's Wonder platform to find mortality and morbidity rates for Influenza, COVID-19, and Measles. In the case of mortality, this specifically used searches based on the International Classification of Diseases (ICD) codes for those specific diseases and did not include related diseases that are comorbid, such as pneumonia in the case of COVID-19 or Influenza. Historical data, pre-1960, was gathered using historical national public health record for the United States from a variety of academic publishers, who have digitally preserved and archived these records. After plotting the initial data, demarcation lines were added to make positive trends clear for increased legibility. Also, bold colors and graphical variety were used to increase visibility, impact, and interest, promoting engagement with the data and associated presenter.



- [VAERS](#)
- [CDC Mortality Data](#) and [CDC](#) Hospitalization rates
- [CDC FLUView Site](#)
- [CDC NSSP Emergency Dept. Visit Trajectories](#)
- [Measles Cases and Outbreaks | Measles \(Rubella\) | CDC](#)
- [MMWR Summary of Notifiable Diseases, United States, 1993](#)

Graphics created show the impact of vaccines on health outcomes, creating a narrative that vaccines can help decrease infection rates, decrease mortality rates or both. In the case of Measles data, the vaccine impact is most stark.



An underprioritized role of public health professionals is the creation of data visualizations that are easy to understand and quick to convey information to the public. While many cater information for medical providers or other public health professionals, tools to assist the general public's health literacy are vital, especially in the face of current events that affect the overall health system, public morale, and public trust towards all health professionals.

1. United States Census Bureau. *Vital Statistics, Statistical Abstract of the United States: 1924 to 1952*. <https://www.census.gov/library/publications/time-series/statistical-abstracts.html>
2. Our World in Data. "Death rate" [dataset] and "Population" [original data]. <https://ourworldindata.org/grapher/measles-cases-and-death-rate>
3. Purcell N, Usman H, Woodruff N, et al. When clinicians and patients disagree on vaccination: What primary care clinicians can learn from covid-19 vaccine hesitancy about communication, trust, and relationships in Healthcare. *BMC Primary Care*. 2024;25(1). doi:1186/s12875-024-02665-4
4. Sasse K, Mahabir R, Gwumtson O, Crooks A, Crofton A. Understanding the determinants of vaccine hesitancy in the United States: A comparison of social surveys and social media. *PLoS ONE*. 2024;19(6). doi:11371/journal.pone.0301488
5. Sandlin EW. The politicization of influenza: Partisan changes in flu vaccination before and after COVID-19. *Journal of Public Health*. Published online February 27, 2025. doi:10.1093/pubmed/dfaf004
6. Nwachukwu G, Rihan A, Nwachukwu E, Uduana N, Elliott KS, Tiruneh YM. Understanding covid-19 vaccine hesitancy in the United States: A systematic review. *Vaccines*. 2024;12(7):747. doi:10.3390/vaccines12070747
7. Enaymaja JC, Anderson SA, Kelly NC, Long M, Duncan CG. The social ecology of health beliefs and misinformation framework: Examining the impact of misinformation on vaccine uptake through individual and sociological factors. *Vaccine*. 2024;42(3):455-463. doi:10.1016/j.vaccine.2024.01.001

Name: Hanko, Diana

Site: Henry Ford Health - Department of Family Medicine

Title: Optimizing Care of Type-2 Diabetes and Obesity with Collaborative Interventions

Introduction: Henry Ford Health System aims to improve outcomes for patients with Type 2 Diabetes (T2D) and comorbid overweight/obesity in underserved Detroit, Michigan. This initiative targets systemic barriers such as limited resources and lack of access to care by integrating a community healthcare worker. This role connects communication and healthcare issues and delivers resources for support. In partnership with a multidisciplinary team, including providers, pharmacists, health educators, and transformation consultants, this intervention enhances medication management and lifestyle management to increase treatment adherence and achieve early glucose control.

Methods: A literature review was created to examine the effectiveness of community healthcare workers in supporting patients with diabetes, obesity, or chronic diseases, as well as identifying programs that offer valuable lessons. Additionally, a comprehensive report was created to examine gaps in healthcare access and care and detail resources, actionable recommendations, and interventions that a community healthcare worker could use to assist patients.

Outcomes: The intervention aims for early glucose control and sustainable weight loss by connecting patients with affordable, appropriate resources and care. Evidence-based strategies include medication management, lifestyle modification counseling, behavioral support, and community healthcare worker assistance. Evaluation metrics include Hemoglobin A1c (HbA1C) levels, body mass index (BMI), patient intervention adherence, patient-reported outcomes, satisfaction, integration of therapeutic recommendations, and care pathway compliance. Successful implementation will not only improve clinical metrics but also enhance patient quality of life and system-wide efficiency.



Name: Kojku, Klea

Site: Michigan Department of Health and Human Services - Office of the Chief Medical Executive

Title: Expanding Access to Reproductive Health Through Free Contraception Resources in Michigan

Introduction: The Michigan Department of Health and Human Services (MDHHS) addresses key public health priorities across the state, including reproductive health access. The Office of the Chief Medical Executive supports initiatives that improve health equity and empower individuals to make informed decisions. One such effort is the Take Control of Your Birth Control program, which expands access to free contraception and family planning resources statewide.

Methods: I supported reproductive health equity by contributing to outreach and communication strategies for the Take Control of Your Birth Control initiative. My responsibilities included:

1. Researching areas with limited access to contraception and identifying community partners.
2. Assisting in the development of messaging to promote the availability of free contraceptives and emergency contraception.
3. Supporting efforts to inform uninsured individuals about coverage options through Medicaid and the Plan First! program.

These activities helped strengthen communication between MDHHS and Michigan residents while increasing the visibility of the initiative.

Outcomes: The Take Control of Your Birth Control program currently supports over 300 sites statewide, distributing oral birth control, emergency contraception, condoms, and educational materials. The initiative empowers individuals to make informed reproductive choices and improves equitable access to care. My work contributed to the broader MDHHS mission of reducing unintended pregnancies and advancing reproductive justice in Michigan.



WAYNE STATE School of Medicine

Expanding Access to Reproductive Health Through Free Contraception Resources in Michigan

Klea Kojku, Master of Public Health Candidate
Natasha Bagdasarian, MD, MPH, FIDSA, FACP (Chief Medical Executive)
Jared Welehodsky, JD (State Assistant Administrator) and
April Stevens, MPH (Communications Liaison)
Michigan Department of Health and Human Services (MDHHS)

Introduction

The Michigan Department of Health and Human Services (MDHHS) addresses key public health priorities across the state, including reproductive health access. The Office of the Chief Medical Executive supports initiatives that improve health equity and empower individuals to make informed decisions. One such effort is the *Take Control of Your Birth Control* program, which expands access to free contraception and family planning resources statewide.¹

The Take Control of Your Birth Control campaign continues to transform reproductive health access across Michigan, setting a new standard for equitable healthcare initiatives. The campaign's comprehensive approach ensures every Michigander has access to essential resources, education, and support.¹

By focusing on equitable distribution, the Take Control of Your Birth Control campaign ensures that Michiganders, regardless of location or socioeconomic status, have access to essential family planning tools.

Methods

Supported reproductive health equity by contributing to outreach and communication strategies for the Take Control of Your Birth Control initiative.

1. Researching areas with limited access to contraception and identifying community partners.
2. Assisting in the development of messaging to promote the availability of free contraceptives and emergency contraception.
3. Supporting efforts to inform uninsured individuals about coverage options through Medicaid and the Plan First! program.

These activities helped strengthen communication between MDHHS and Michigan residents while increasing the visibility of the initiative.

Conclusion



The Take Control of Your Birth Control program supports over 300 sites statewide, distributing oral birth control, emergency contraception, condoms, and educational materials. The initiative empowers individuals to make informed reproductive choices and improves equitable access to care. This initiative contributes to the broader MDHHS mission of reducing unintended pregnancies and advancing reproductive justice in Michigan.² It is estimated an additional 25,000 individuals could receive resources, which include over-the-counter oral birth control pills, emergency contraception, condoms, and family planning educational information.

Results

- The Take Control of Your Birth Control campaign has successfully distributed over 210,000 critical reproductive health resources, including emergency contraception, oral contraceptives, and condoms, to every county in the state, ensuring no community is left behind. To date, the DHHS has shipped²:
 - 34,300 doses of emergency contraceptives
 - 34,300 doses of oral birth control pills
 - 171,500 condoms
- The campaign's robust educational outreach has increased awareness of insurance options like Plan First and Medicaid, equipping residents with knowledge to make informed reproductive health decisions.²

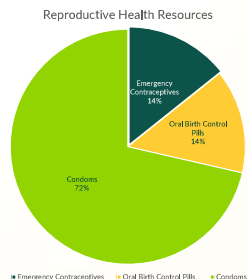


Figure 1 – Reproductive Health Resources Distribution



Figure 2 – Plan First Brochure



Figure 3 – Insurance Coverage Brochure

References

1. Michigan.gov/TakeControl
2. Cwiek, S. (2024, March 14). State offers free birth control at 300 Michigan locations. Michigan Advance. <https://michiganadvance.com/briefs/state-offers-free-birth-control-at-300-michigan-locations>

Name: Latawiec, Amy Ann

Site: Corewell Health East

Title: Supporting the Racial and Ethnic Approaches to Community Health (REACH) grant in its first year

Introduction: Corewell Health East is a part of Corewell Health, a non-profit health system in Michigan. In 2023 Corewell Health received the Racial and Ethnic Approaches to Community Health (REACH) grant. In its first year, Corewell Health is building the community coalition for REACH in southeast Michigan with the goal of creating local programs to address a wide range of health disparities. As Corewell Health builds this coalition, they were in need of support for three projects: data analysis, focus group development, and landscape analysis research.

Methods: The first project was to design focus groups to gather community feedback regarding health disparities in the areas of nutrition, physical fitness, mental health, and breastfeeding. The second project was qualitative analysis for a school pantry program. The third project was the research and development of initial action plans for a landscape analysis of the target communities in the REACH grant: River Rouge, Inkster, East Detroit, and West Detroit

Outcomes: I completed a presentation on focus group design and implementation including a set of questions for focus group sessions. I utilized qualitative thematic analysis to write a stakeholder analysis and recommendations report on the efficacy of the school pantry program based on qualitative data collected by program participants. To launch the landscape analysis for the REACH grant, I provided guidance for the team at Corewell on the utilization of each city's Master Plan documents for initial data collection. I gave presentations at team meetings on how to utilize each city's Master Plan documents.



Supporting the R.E.A.C.H. grant in its first year

Amy Latawiec, PhD, Master of Public Health Candidate
Kari Woloszyk, MPH CHES, Business manager and site preceptor
Corewell Health East

Introduction

Corewell Health East is a part of Corewell Health, a non-profit health system in Michigan. In 2023 Corewell Health received the R.E.A.C.H. (Racial and Ethnic Approaches to Community Health) grant.

In its first year, Corewell Health is building the community coalition for R.E.A.C.H. in southeast Michigan with the goal of creating local programs to address a wide range of health disparities. As Corewell Health builds this coalition, they were in need of support for three projects: data analysis, focus group development, and landscape analysis research.

Methods

The **first project** was to design focus groups to gather community feedback regarding health disparities in the areas of nutrition, physical fitness, mental health, and breastfeeding.

The **second project** was qualitative analysis for a school pantry program.

The **third project** was the research and development of initial action plans for a landscape analysis of the target communities in the R.E.A.C.H. grant: River Rouge, Inkster, East Detroit, and West Detroit.

Outcomes

- I completed a presentation on focus group design and implementation including a set of questions for focus group sessions.
- I utilized qualitative thematic analysis to write a stakeholder analysis and recommendations report on the efficacy of the school pantry program based on qualitative data collected by program participants.
- To launch the landscape analysis for the R.E.A.C.H. grant, I provided guidance for the team at Corewell on the utilization of each city's Master Plan documents for initial data collection. I gave presentations at team meetings on how to utilize each city's Master Plan documents.

Conclusion

Joining Corewell Health East in their first year of implementing the R.E.A.C.H. grant created several opportunities to support the staff at Corewell Health as they build the community coalitions to positively impact several health indicators in Southeast Michigan communities. Through qualitative research methods consulting as well as research into multiple cities master plan documents, Corewell Health is now better prepared to collect qualitative data, communicate those results effectively, and begin their landscape analysis for the physical fitness pillar of the R.E.A.C.H. grant.

Deliverables Grid

Public Health Competency	Deliverable 1 – Completion of Active Communities Tool	Deliverable 2 - Focus Group Design	Deliverable 3 – Data analysis and stakeholder recommendations
Design a population-based policy, program, project, or intervention (Foundational - 9)		X	
Advocate for political, social, or economic policies and programs that will improve health in diverse populations (Foundational - 14)	X	X	X
Evaluate policies for their impact on public health and health equity (Foundational - 15)	X	X	
Propose public health interventions that reflect the social determinants of health for a given community (Core -2)	X		X
Synthesize current public health evidence with community and population assessment (Core - 3)	X		X

Name: Lund, Mallory

Site: Integrative (IBio) Biosciences Center - Behavioral Research Lab

Title: A descriptive study on sociodemographic exposures and drug use

Introduction: Alcohol use has historically shown a male-excess, yet recent reports suggest females may outpace their male counterparts. Further, while living in a rural area has been associated with greater substance use, it is unknown whether patterns vary by biological sex. We aim to estimate the relationship between incident alcohol use and biological sex, and to test effect modification by urbanicity.

Methods: The target population of the 2023 National Survey on Drug Use and Health (NSDUH) survey is non-institutionalized US civilians aged 12+ years; analytic sample was made up of participants who either 1) used alcohol for the first time within the 12-24 months prior to survey, or 2) never used alcohol. Survey weighted logistic regression produced odds ratio (ORs) and 95% confidence intervals (CIs); effect modification by urbanicity was tested using an interaction term. Variance estimation was based on a Taylor series linearization; statistical analysis was performed using R Studio (2024.12.1 Build 563) software.

Outcomes: The sample of 19,103 participants surveyed resulted in a weighted 65,513,597 participants aged 12+ years. Past 12–24-month incident alcohol use was estimated at 10.17% of participants (95% CI: (10.16%, 10.17%). By sex, 5.10% of females (95% CI: (5.09%, 5.10%)) and 5.06% of males (95% CI: (5.06, 5.07)) began use. Estimates between incident alcohol use and biological sex were null (OR=1.14; 95% CI: (0.99, 1.30)) as well as urbanicity (p=0.31).



A descriptive study on sociodemographic exposures and drug use

Mallory A. Lund, BA

Preceptor: Samantha J. Bauer, PhD, MS, Assistant Professor, Department of Family
Medicine and Public Health Sciences
Wayne State University School of Medicine
Wayne State University Integrative Biosciences Center

Introduction

Alcohol use historically shows a male-excess, yet recent reports suggest females may outpace their male counterparts. Further, while living in a rural area has been associated with greater substance use, it is unknown whether patterns vary by biological sex. We aim to estimate the relationship between incident alcohol use and biological sex, and to test effect modification by urbanicity.

Methods

The target population of the 2023 National Survey on Drug Use and Health (NSDUH) survey¹ is non-institutionalized US civilians aged 12+ years; analytic sample was made up of participants who either 1) used alcohol for the first time within the 12-24 months prior to survey, or 2) never used alcohol. Survey weighted logistic regression produced odds ratio (ORs) and 95% confidence intervals (CIs); effect modification by urbanicity was tested using an interaction term. Variance estimation was based on a Taylor series linearization; statistical analysis was performed using R Studio (2024.12.1 Build 563) software.

Results

Figure 1 – Sample Characteristics

Characteristics	Unweighted (N=19,103) n	%	Weighted (N=65,513,597) n	Point Estimate (%)	95% Confidence Interval
Sex					
Female	9,775	51.17%	35,208,118	53.74%	(53.72, 53.75)
Male	9,328	48.83%	30,305,479	46.25%	(46.24, 46.27)
Race/ethnicity					
Non-Hispanic white	7,866	41.18%	29,027,600	44.30%	(44.29, 44.31)
Non-Hispanic Black/African American	3,004	15.73%	10,917,818	16.66%	(16.65, 16.67)
Non-Hispanic Native American/Alaskan Native	619	3.24%	475,477	0.72%	(0.72, 0.72)
Non-Hispanic Native Hawaiian/Other Pacific Islander	140	0.73%	397,550	0.60%	(0.60, 0.60)
Non-Hispanic Asian	1,277	6.68%	7,238,901	11.04%	(11.04, 11.05)
Non-Hispanic more than one race	1,074	5.62%	1,581,574	2.41%	(2.41, 2.41)
Hispanic	5,123	26.82%	15,874,673	24.23%	(24.22, 24.24)
Age					
12-17	10,133	53.04%	22,952,839	35.03%	(35.02, 35.04)
18-25	4,937	25.84%	12,427,529	18.96%	(18.95, 18.97)
26-34	1,180	6.18%	4,875,883	7.44%	(7.43, 7.44)
35-49	1,525	7.98%	8,776,116	13.39%	(13.38, 13.40)
50+	1,328	6.95%	16,481,231	25.15%	(25.14, 24.16)
Urbanicity					
Large metro	8,874	46.45%	36,271,299	55.36%	(55.35, 55.37)
Small metro	7,274	38.08%	20,975,380	32.01%	(32.00, 32.02)
Nonmetro	2,955	15.47%	8,266,918	12.61%	(12.61, 12.62)
Education					
Less than high school	1,843	9.65%	8,227,095	12.55%	(12.54, 12.56)
High school graduate	3,421	17.91%	15,215,474	23.22%	(23.21, 23.23)
Some college/Assoc Degree	2,141	11.21%	10,297,003	15.7135%	(15.70, 15.72)
College Graduate	1,565	8.19%	8,821,186	13.46%	(13.45, 13.47)
12-17 Yr olds (null)	10,133	53.04%	22,952,839	35.03%	(35.02, 35.04)
Family Income					
<\$20,000	3,652	19.12%	12,589,824	19.21%	(19.20, 19.22)
\$20,000-\$49,999	5,639	29.52%	19,019,057	29.03%	(29.01, 29.04)
\$50,000-\$74,999	2,565	13.43%	8,809,011	13.44%	(13.43, 13.45)
\$75,000+	7,247	37.94%	25,095,704	38.30%	(38.29, 38.31)

Figure 2 – Regression model results							
	Model 1			Model 2		Model 3	
	OR	95% CI		OR	95% CI	OR	95% CI
Biological Sex	Males	Ref.	--	Ref.	--	Ref.	--
	Females	1.13	(0.99, 1.31)	1.18	(1.03, 1.35)	1.14	(0.99, 1.3)
Age Category	18-25	3.5	(3.05, 4.03)	1.41	(1.05, 1.9)	1.4	(1.04, 1.8)
	26-34	0.26	(0.12, 0.56)	0.13	(0.05, 0.34)	0.13	(0.05, 0.3)
	35-49	0.11	(0.05, 0.26)	0.07	(0.03, 0.16)	0.07	(0.03, 0.1)
	50+	0.005	(0.001, 0.02)	0.01	(0.0, 0.02)	0.0	(0, 0.0)
Race/Ethnicity	Non-Hispanic Black/African American	0.69	(0.56, 0.84)	0.72	(0.58, 0.91)	0.74	(0.59, 0.9)
	Non-Hispanic Native American/Alaskan Native	0.84	(0.42, 1.67)	1.06			
	Non-Hispanic Native HI/Other Pac. Islander	0.41	(0.13, 1.27)	0.39	(0.12, 1.33)	0.38	(0.11, 1.2)
	Non-Hispanic Asian	0.67	(0.47, 0.96)	0.73	(0.5, 1.07)	0.76	(0.52, 1.1)
	Non-Hispanic more than one race	1.24	(0.90, 1.71)	1.33	(0.96, 1.84)	1.3	(0.93, 1.8)
	Hispanic	0.90	(0.74, 1.08)	0.95	(0.78, 1.18)	0.96	(0.78, 1.1)
Education	High school graduate	--	--	1.30	(0.94, 1.81)	1.30	(0.93, 1.8)
	Some college/Assoc Degree	--	--	1.74	(1.26, 2.41)	1.75	(1.26, 2.42)
	College Graduate	--	--	1.10	(0.61, 2.00)	1.12	(0.62, 2.0)
Marital Status	Widowed	--	--	0.50	(0.05, 4.77)	0.48	(0.05, 4.4)
	Divorced or separated	--	--	0.24	(0.05, 1.17)	0.25	(0.05, 1.2)
	Never been married	--	--	1.59	(0.98, 2.6)	1.57	(0.96, 2.57)
	Respondent <14 yrs old (null)	--	--	0.28	(0.15, 0.55)	0.28	(0.14, 0.54)
Income	\$20,000 - \$49,999	--	--	4.77	(0.05, 4.77)	1.18	(0.95, 1.47)
	\$50,000 - \$74,999	--	--	1.17	(0.05, 1.17)	1.1	(0.89, 1.3)
	\$75,000 or More	--	--	2.60	(0.98, 2.6)	1.44	(1.16, 1.7)
	Null	--	--	0.55	(0.15, 0.55)	--	--
Employment Status	Employed part time	--	--	0.86	(0.65, 1.16)	0.85	(0.63, 1.1)
	Unemployed	--	--	0.79	(0.58, 1.09)	0.78	(0.57, 1.0)
	Other (incl not in labor force)	--	--	0.52	(0.41, 0.67)	0.51	(0.4, 0.65)
Overall Health Rating	Very good	--	--	--	--	1.41	(1.14, 1.7)
	Good	--	--	--	--	1.58	(1.27, 1.9)
	Fair/poor	--	--	--	--	1.59	(1.16, 2.1)

Conclusion

- The analytic sample of 19,103 participants resulted in a weighted sample of 65,513,597 participants aged 12+.
- Incident alcohol use within 12-24 months of begin surveyed was observed among 10.17% of participants (95% CI: (10.16%, 10.17%).
- Stratified by sex, 5.10% of females (95% CI: (5.09%, 5.10%) and 5.06% of males (95% CI: (5.06, 5.07) started using alcohol in the past year.
- Estimates between incident alcohol use and biological sex were null (OR=1.14; 95% CI: (0.99, 1.30)).
- The relationship between incident alcohol use and biological sex was not effect modified by urbanicity (p=0.31).

References

- Substance Abuse and Mental Health Services Administration. Download NSDUH Data Files. SAMHSA. Published February 13, 2025. Accessed February 1, 2025. <https://www.samhsa.gov/data/data-we-collect/nsduh-national-survey-drug-use-and-health/datafiles>

Name: McParland, Tania

Site: J.O.B. Lactation Foundation

Title: Addressing Postpartum Depression Through Doula Support: Insights from a Community-Based Needs Assessment

Introduction: Postpartum depression affects 1 in 7 women and disproportionately impacts underserved communities with limited access to emotional and mental health support. This practicum project explores how doula care – a non-clinical, supportive service – can help reduce symptoms of postpartum depression by providing emotional check-ins, infant care support, and in-home visits.

Methods: A needs assessment was conducted through a survey of five postpartum individuals. Participants were asked about their experiences with emotional support, familiarity with doula services, and what support they would want in the future. Responses were analyzed to identify common themes and areas of need.

Outcomes: All five participants reported emotional challenges postpartum and expressed interest in doula services. Key areas of desired support included emotional check-ins, infant care and feeding guidance, in-home visits, and having someone to talk to. The findings support the development of a postpartum doula program to improve maternal health outcomes in underserved communities.



Addressing Postpartum Depression Through Doula Support: Insights from a Community-Based Needs Assessment

Tania McParland, BS, Master of Public Health Candidate
Preceptor: Micia Eddins, MPH, IBCLC, CD, CHW, CLS, Nutrition Specialist
J.O.B Lactation Foundation

Introduction

J.O.B (Joys of Breastfeeding) Lactation Foundation is a nonprofit organization offering in-person and virtual doula and lactation services to families from pregnancy through the first year of the infant's life. Services include prenatal and postpartum support, breastfeeding education, tongue-tie assessments, and peer-led support group sessions.

Postpartum depression (PPD) affects 1 in 7 women in the U.S and can have serious consequences for both maternal and infant health. The emotional and physical toll of childbirth – compounded by lack of sleep, changing identity, and social isolation – can make the postpartum period especially vulnerable. These risks are even greater for mother in underserved communities, where access to affordable and culturally responsive care is limited.

Doulas offer consistent, non-clinical support that has been shown to reduce stress, improve confidence, and build a trusted relationship during the perinatal period. Emotional check-ins, in-home visits, and infant care guidance from trained doulas may serve as a protective factor against postpartum depression. However, many families are unaware of doula services or face barriers to accessing them, such as cost, stigma, or lack of availability in their area.

Many mothers, especially those from underserved communities, lack access to consistent emotional and physical support during the perinatal period.

This project focuses on identifying gaps in postpartum mental health support and assessing community needs to guide the development of a postpartum doula program. By collecting first-hand data from new mothers, this practicum aims to build a model that expands access to maternal support services and addresses the emotional and mental health needs of at-risk parents in a meaningful, sustainable way.

Objective:

Identify community needs and barriers to accessing mental health and postpartum support.

Increase access to postpartum support services for at-risk populations.

Goal: Identify gaps in postpartum support and determine how a doula program can effectively reduce postpartum depression. Use this data to tailor the program to meet specific needs of the community.



Methods

Need assessment survey:

- Qualtrics survey for prior clients of the J.O.B Lactation Foundation.
- Target population was parents especially within 1 year postpartum.

Survey:

- Qualtrics survey
- Participants were prior clients of the foundation.
- 6 individuals initiated the survey but only 5 completed the survey.
- Sample 4 black women and 1 other woman of childbearing age. N=5

Key Survey sections included:

- Demographics
- Pregnancy and postpartum experience
- Mental health experience
- Awareness and use of doula services
- Support needs such as a doula would be considered if one had been available at low or no cost.

Results

- Results of the survey (n=5)
- 100% of participants reported experiencing some level of emotional or mental health challenges during the postpartum period, including stress, anxiety, or feelings of isolation.
- 4 out of 5 participants said they did not receive any professional emotional support after giving birth.
- All participants (5/5) expressed interest in having access to a postpartum doula if one were available to them.
- When asked about specific types of support they would find helpful:
- 5/5 selected emotional check-ins
- 4/5 selected infant care and feeding guidance.
- 4/5 selected in-home visits
- 5/5 selected having someone to talk

"I felt so alone after having my baby. Just having someone check in on me or help with feeding would've made a huge difference."
— Survey respondent, postpartum mother



Figure 1

Conclusion

Survey outcome:

Although the sample size was small (n=5), the survey revealed a clear need for increased postpartum support. All participants reported experiencing stress, anxiety or emotional challenges after childbirth, and most indicated that they were unaware of or unable to access doula services. Participants expressed strong interest in receiving emotional support, breastfeeding guidance, and help navigating postpartum changes.

These findings suggest that even a limited group of new parents perceive a gap in care that a postpartum doula program could fill – underscoring the value of expanding access to trained, culturally responsive doulas in the community.

Recommendations provided to the J.O.B Lactation Foundation:

- Emotional and mental health check-ins in the early weeks after delivery.
- Hands-on guidance with infant care, breastfeeding and sleep routines.
- In-home or mobile support to reduce the burden of traveling with a newborn.
- Someone to talk to who is trained, non-judgmental, and understands the postpartum experience.

References

- Gruber, K. J., Cupito, S. H., & Dobson, C. F. (2013). Impact of doulas on healthy birth outcomes. *J Perinat Educ*, 22(1), 49–58.
- Kozhimannil, K. B., Attanasio, L. B., Hardeman, R. R., & O'Brien, M. (2013). Doula care, birth outcomes, and costs among Medicaid beneficiaries. *Am J Public Health*, 103(4), e113–e121
- McLeish, J., & Redshaw, M. (2017). Mothers' accounts of the emotional impact of peer support in pregnancy and early parenthood. *BMC Pregnancy Childbirth*, 17, 28
- Kozhimannil, K. B., Vogelsang, C. A., Hardeman, R. R., & Prasad, S. (2016). Disrupting social determinants of health: Doula support during pregnancy and childbirth. *J Am Board Fam Med*, 29(3), 308–317
- Lantz, P. M., Low, L. K., Varkey, S., & Watson, R. L. (2005). Doulas as childbirth paraprofessionals: Results from a national survey. *Women's Health Issues*, 15(3), 109–116

Name: Murphy, Kristin

Site: Damon J. Keith Center for Civil Rights, Wayne State University School of Law

Title: Combatting Spatial Structural Racism: Examining Intersectionality of Public Health, Civil Rights, and Socio-economic Support in Detroit, Michigan

Introduction: Spatial Structural racism is a root cause of health disparities and socio-economic inequities in Detroit. Nonprofits commonly provide essential services such as housing, food, and healthcare to meet the basic needs of residents. Without understanding the historical context of Detroit and how spatial structural racism causes negative outcomes to produce, reproduce, and silo the nonprofit service network; providers lack a holistic, rights-based approach that addresses the rights violations that marginalize residents. This study explores nonprofit providers' perspectives on these intersections and informs a training series designed to equip participants with the knowledge and tools to address structural racism through systems thinking and collaboration.

Methods: A survey was created to evaluate how nonprofit service providers perceive their roles in relation to public health, civil rights, and human rights and their awareness of spatial structural racism. Following the survey, a three-part training series was drafted to enhance nonprofit service provider's understanding of the intersectionality between public health, civil rights, and human rights in Detroit, with a focus on spatial structural racism and systems thinking.

Outcomes: The survey revealed a gap in how service providers perceive the historical context of Detroit and its impact on their work. These gaps contribute to nonprofit providers identifying themselves as civil or human rights advocates and members of the public health workforce; yet often without a shared framework connecting these role and resources. The three-part training series will strengthen the nonprofit workforce by deepening understanding of the intersections between public health, civil rights, and human rights in Detroit.



Combatting Spatial Structural Racism: Examining Intersectionality of Public Health, Civil Rights, and Socio-economic Support in Detroit

Kristin M. Murphy, BA, Master of Public Health Candidate
Peter J. Hammer, J.D., Ph.D., Director of Damon J. Keith Center for Civil Rights

Background

- Spatial structural racism is a root cause of health disparities and socio-economic inequities in Detroit.
- In response to these inequities nonprofits often provide essential services such as housing, food, and healthcare—to meet the basic needs of residents.
- Lack of Detroit historical context limits providers' ability to recognize spatial structural racism and prevents a holistic, rights-based approach.
- This study examined the perspectives of nonprofit service providers on the intersections between civil rights, human rights, and public health to inform a training series designed to create a deeper understanding of structural racism.



Figure 1: Evolution of Structural Spatial Racism

Methods

Assessment

- A survey was created to assess how nonprofit service providers perceive their roles in relation to public health, civil rights, and human 24-questionionion
- The survey consisted of twenty-four questions and four sections: demographics, civil and human rights impact on public health, current practices and needs, and collaboration and systems thinking.

Training Materials

- A three-part training series was created to enhance the understanding of the intersectionality between public health, civil rights, and human rights to combat Detroit Spatial Racism.

Session 1: Understanding the Landscape

Session 2: How to Combat Spatial Structural Racism in Detroit with Systems Thinking

Session 3: Building a Nonprofit Systems Thinking Network

Outcomes

Survey Findings

- A gap was found in service providers' understanding of Detroit's historical context and its relevance to their work.
- Nonprofit workers are not commonly trained to view themselves as civil rights and human rights advocates and members of the public health workforce.

Training Goals

- Enhance providers' understanding of the intersections between public health, civil rights, and human rights in Detroit
- Focus on spatial structural racism as a root cause of inequity
- Equip service providers with foundational knowledge and tools to identify and address inequities within their work

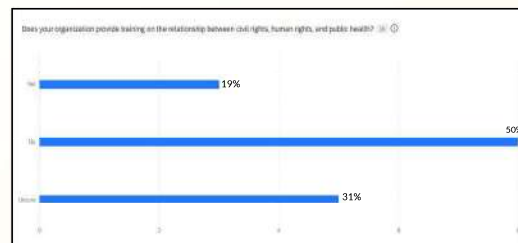


Figure 2: Survey results for question "Does your organization provide training on the relationship between civil rights, human rights, and public health?"

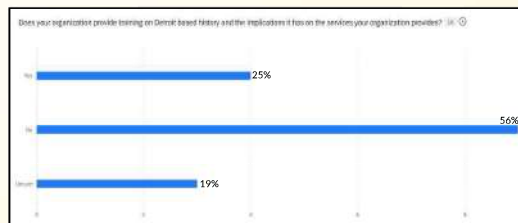


Figure 3: Survey results for question "Does your organization provide training on Detroit based history and the implications it has on the services your organization provides?"

Public Health Implications

- Public health, civil rights, and human rights have overlapping goals and methods that have deep historical context.
- These intersections must be understood by those who provide advocacy and resource-based services for work to be effective and equitable.
- Cross-sector collaboration, particularly through targeted training, can strengthen nonprofit services in Detroit by aligning approaches across public health, civil rights, and human rights.



Figure 4: Venn Diagram of Intersections between civil rights, human rights, and structural racism

References

1. American Medical Association. (2021). What is structural racism?. American Medical Association. <https://www.ama-assn.org/delivering-care/health-equity/what-structural-racism>
2. Bekkering, H., & Liu, Y. (2015). Mapping Detroit "The City of Holes.". Mapping Detroit Land, Community, and Shaping a City, 27-48.
3. City Snapshot: Detroit. Othering & Belonging Institute. (n.d.). <https://belonging.berkeley.edu/city-snapshot-detroit>
4. Hammer, P. J. (2018). Detroit 1967 and today: spatial racism and ongoing cycles of oppression. J.L Soc'y, 18, 227.
5. Mills, C. (2007). White ignorance. Race and epistemologies of ignorance, 247, 26-31.
6. Roots of structural racism. Othering & Belonging Institute. (2020). <https://belonging.berkeley.edu/roots-structural-racism-2020>
7. YouTube. (n.d.). YouTube. <https://www.youtube.com/watch?v=2a7yn1kTy4E>

Name: Oliver, Boston

Site: Coalition on Temporary Shelter (COTS) Detroit

Title: Bridging the Food Access and Security Gap: A Community-Based Approach to Supporting Families in Supportive Housing

Introduction: Coalition on Temporary Shelter (COTS) is an organization committed to providing housing assistance and shelter for families facing or experiencing homelessness. Peterboro Arms Apartments is the supportive housing facility which provides long-term supportive housing for 56 families overcoming homelessness. COTS provides an array of services across several domains through its transformative coaching program.

Methods: To bridge the gap between available food resources and the needs of the families at Peterboro Arms Apartments, I developed a needs-based assessment, adapting it from the U.S. Household Food Security Survey. The assessment included targeted questions designed to understand residents' food access and security challenges, as well as their needs and preferences for improving food access to be implemented by COTS. The survey was administered onsite at Peterboro Arms and remotely via Microsoft Teams to ensure participation. In addition, I researched how COTS could become a viable food distribution site for food assistance for residents and the community. Following, I developed a food distribution site map identifying the most accessible locations near Peterboro Arms that can be accessed by families to increase awareness of local food resources.

Outcomes: The needs-based assessment identified the challenges faced by residents and gathered their suggestions for how COTS can improve food access, ultimately impacting food security. A summary report was created to share the findings with leadership and staff.



Bridging the Food Access and Security Gap: A Community-Based Approach to Supporting Families in Supportive Housing

Boston Oliver, Master of Public Health and Master of Social Work Candidate
Nikki Carbonari, LMSW, Executive Director of Impact

Introduction

Coalition on Temporary Shelter(COTS) is a non-profit organization founded to address the city's growing homelessness crisis.¹ Since its founding in 1982, COTS has provided a wide range of support services to families overcome homelessness such as permanent supportive housing.

COTS utilizes the Passport to Self-Sufficiency framework to empower families toward long-term stability.² This framework is centered around mobility coaching focused on housing and family, health and well-being, economic mobility and empowerment, education and job training, as well as employment and career development.

My work with COTS was centered at Peterboro Arms, the supportive housing facility designed to provide long-term housing for families. I focused on understanding how COTS can successfully bridge the gap between food resources and the needs of families in supportive housing. By addressing food insecurity, COTS can help improve the overall outcome of its residents. This work contributes to the vital effort of fostering healthier communities.

Methods

- ❖ Researched needs-based assessment developed by others to understand key components and methodologies that would be effective for this project.
- ❖ Developed an assessment survey with questions targeting food access and security challenges.
- ❖ Distributed the assessment survey to residents through digital and paper formats as well as offered completion incentives.
- ❖ Analyzed and interpreted the results from the assessment survey.
- ❖ Explored successful food programs and innovated approaches implemented in other cities and states to identify the best practices.
- ❖ Researched local food banks and community organizations to explore a potential partnership as a food distribution site.
- ❖ Developed a food distribution site map for residents use.
- ❖ Collaborated and received feedback from preceptor to ensure alignment with COTS mission needs of residents.
- ❖ Developed an executive summary report outlining the results and propose recommendations for improving food access based on responses.

Deliverables

- ❖ Developed and dispersed a needs-based assessment to understand residents' food access and security challenges, as well as their preferences for improving food access to be implemented by COTS.
- ❖ Developed a food distribution site map to identify locations near Peterboro Arms to be shared with families to increase awareness of local food resources.



Food Distribution Sites Near You



COTS- Peterboro Arms	Distribution Locations
- Unify Detroit/ St. John Presbyterian Church 1961 E Lafayette St Detroit 48207 3rd Friday 9 AM- 11 AM - Franklin Wright Settlements 2560 Charlevoix St Detroit 48207 Mondays 3 PM - Genesis Hope 7200 Mack Ave Detroit 48214 2nd & 3rd Wednesday 2:30 PM- 4 PM - Church of God Baldwin 5540 Talbot St Hamtramck 48212 Wednesdays 11 AM- 5 PM - Love Joy Church of God in Christ 17903 Greeley Highland Park 48203 Thursdays & Saturdays 1:30 PM - Prayer Temple of Love 12375 Woodward Ave Highland Park 48203 2nd Monday 9 AM- 12 AM - Historic Little Rock Ministries 8000 Woodward Ave Detroit 48208 2nd & 4th Friday 9 AM- 12 PM - Metropolitan United Methodist 800 Woodward Ave Detroit 48203 Fridays 2 PM- 8 PM	- Central Detroit Christian 1550 Taylor St Detroit 48226 Mondays 2 PM- 4 PM - Shrine of the Black Madonna 7625 Linwood St Detroit 48206 1st Saturday 11 AM - Valley Temple Church of God in Christ 5370 McKinley St Detroit 48208 Tuesdays 12:30 PM- 4 PM - Alternative for Girls 945 W Grand Blvd Detroit 48208 Sat & 3rd Thursday 9 AM- 11 AM - All Saints Literary Center 3553 W Vernor Hwy Detroit 48216 Thursdays 9 AM- 11 AM - Church of Christ Macedonia 6025 Woodrow Street Detroit 48210 2nd & 4th Tuesday 1:30 PM- 3:30 PM - Salvation Army- Temple Corps 3728 W Chicago Detroit 48206 Thursdays 10 AM- 11:30 AM - Triumph Church- East 2760 E Grand Blvd Detroit 48211 Sat & 3rd Fridays 10 AM - Linwood Church of Christ 14901 Linwood St Detroit 48208 2nd Saturdays 12 PM- 2 PM

Figure 2- Food Distribution List

Results

- ❖ Received 27 responses, 4 were not included in the data due to inconsistencies within responses, and 3 were not included due to duplicate responses.
- ❖ 63% reported that within the last 30 days they had cut the size of meals or skipped meals due to financial constraints.
- ❖ 73% reported experiencing this issues on some days but not every day.
- ❖ 47% reported that they were hungry but there was not enough food benefits available to buy food.
- ❖ 53% reported that they were aware of food resources, but 48% indicated that they were not, suggesting some potential barriers.
- ❖ Food access recommendations included possible partnerships with local grocery stores, a food pantry with extended hours within their building, frequent food distribution events, assistance with food benefits, transportation to local food pantries, recipes for budgets.

Conclusion

The findings highlight both challenges and opportunities in improving food access and food security for Peterboro Arms residents. Implementing food resource strategies will help to significantly improve food security for residents. These efforts will also advance the mission of COTS by continuing to promote long-term stability.

Next Steps

- ❖ Establish a *little free pantry* in the Peterboro Arms community room during the summer of 2025 to provide easily accessible non-perishable food items.³ This initiative will be developed in collaboration with COTS mobility coaches and volunteers, serving as the first step toward becoming a food distribution partner.
- ❖ The food distribution map will be implemented by integrating it into the new resident's welcome packet and posting it the elevators of Peterboro Arms.

References

1. About Us—COTS. (n.d.). COTS. <https://cotsdetroit.org/about-us/>
2. Passport To Self Sufficiency - 1223 | COTS. (2023, December 21). Cotsdetroit.org. <https://cotsdetroit.org/passport-to-self-sufficiency-1223/>
3. little free pantry. (n.d.). Little Free Pantry. <https://www.littlefreepantry.org/>

Name: Raslan, Kareem

Site: BLARE

Title: Empowered Online: Creating a Digital Health Literacy Curriculum for Patients with Chronic Conditions

Introduction: The world faces an aging population who have found trouble in adapting to a post-internet world. Lack of understanding in how to use their digital devices and trouble navigating online misinformation is resulting in a population that are finding themselves lost in a society that puts greater value on digital literacy and ability. The effect of this valuation can be seen in all aspects of life, but is especially prevalent in the field of health, as healthcare systems in the United States have largely moved to online spaces.

Methods: To develop a comprehensive digital health literacy curriculum, a literature review was conducted, analyzing peer-reviewed articles on digital literacy interventions and validated assessment measures. A six-module curriculum was designed to equip clients with digital skills essential for navigating online health resources. Modules cover key competencies, including basic device operation, internet navigation, and patient portal access. Practical guidance is emphasized to ensure accessibility for those with limited experience.

Outcomes: As the curriculum is still in development, formal outcome data is not yet available. However, future assessment plans include testing with healthcare guides and evaluating participant progress by assessments. Success will be measured by improvements in digital confidence, navigation skills, and independent online healthcare resource engagement. The goal is to create a scalable and adaptable program that empowers both educators and clients to bridge the digital divide in healthcare access.



Empowered Online: Creating a Digital Health Literacy Curriculum for Patients with Chronic Conditions

Kareem Raslan, Master of Public Health Candidate
Preceptor: Katrina Studvent, MSW, Director of BLARE

Introduction

The world faces an aging population, many of whom struggle to adapt to an increasingly digital society. Older adults often lack the foundational skills necessary to operate digital devices or evaluate the quality of online information, leaving them vulnerable to misinformation and exclusion. Nowhere is this digital divide more consequential than in healthcare, where systems have largely transitioned to online platforms. Tasks such as scheduling appointments, accessing medical records, and communicating with providers now require digital literacy. For individuals unfamiliar with this landscape, navigating their own healthcare becomes not only daunting but, at times, inaccessible. Bridging this gap is essential to ensuring equitable access and participation in modern health systems.

Methods

- To develop a comprehensive digital health literacy curriculum, an extensive literature review was conducted, analyzing articles on digital literacy and digital health literacy interventions. Particular attention was paid to studies employing validated assessment measures, including those evaluating patient portal usage through direct patient-reported experiences and those that assessed patients' self-reported comfort and competency in digital communication, such as navigating telehealth platforms or composing secure messages.
- A six-module curriculum was designed to equip clients with digital skills essential for navigating online health resources. Modules cover key competencies, including basic device operation, internet navigation, and patient portal access. Practical, step-by-step guidance is emphasized to ensure accessibility for those with limited prior experience.

Deliverables

Deliverable 1:
Production of a full curriculum that serves as both a how-to guide for healthcare guides to approach learning how to navigate healthcare technology systems and as a step-by-step teaching process for how to teach patients the same abilities.

Deliverable 2:
A review of the questioning process used to assess digital literacy and digital health literacy. Questions review was aligned with finds from a literacy review of validated measures to revise the existing Participant Questionnaire.

B. Basic Device									
1. I can turn my phone on and off.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
2. I can use my phone to make a call.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
C. Internet Navigation									
3. I can use my phone to go to a website.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
4. I can use my phone to search for information.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
D. Patient Portal Access									
5. I can use my phone to log in to my patient portal.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
6. I can use my phone to send a message to my provider.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
E. Digital Health Literacy									
7. I can understand what my provider says on my phone.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
8. I can use my phone to understand what my provider says.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
9. I can use my phone to understand what my provider says.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never
10. I can use my phone to understand what my provider says.	Never	A few times a week	A few times a month	Once a week	A few times a week	A few times a month	Once a week	A few times a week	Never

Copyright 2018 The Functional Assessment of Currently Employed Technology Skills (FACETS). Charles M. Lepkowski, Ph.D.

Figure 1: FACETS, a validated assessment referenced in developing the post-intervention questionnaire

DHLS Digital Health Care Literacy Scale

1. I can use applications/programs (like Zoom) on my cell phone, computer, or another electronic device on my own (without asking for help from someone else).

2. I can set up a video chat using my cell phone, computer, or another electronic device on my own (without asking for help from someone else).

3. I can solve or figure out how to solve basic technical issues on my own (without asking for help from someone else).

Response options:

- Strongly disagree-0 points
- Disagree-1 point
- Neutral-2 points
- Agree-3 points
- Strongly agree-4 points

Sum score of individual items to create total score. Possible scores range from 0 to 12, with higher scores indicating higher digital health care literacy.

Figure 2: DHLS, a validated assessment by which to measure health skills referenced in developing the post-intervention questionnaire

Outcomes

Pilot testing with healthcare guides have shown the curriculum to be effective in raising confidence in digital health literacy, but that the modules were too wide in scope to be applicable to an entire group of people of varying skills as intended. Further evaluation is still need, and success will be measured by improvements in digital confidence, navigation skills, and the ability to independently engage with online healthcare resources. The goal is to create a scalable and adaptable program that empowers both educators and clients to bridge the digital divide in healthcare access.

Next Steps

Paring down of the modules to be more summarized guides for the healthcare guides to approach the needs of individual clients is necessary. The modules in their current form are best kept as step-by-step handouts to refer back to, allowing the guides to focus on the broad details necessary to cover for a multitude of people.

References

- Lepkowski, Charles & Arndt, Stephan. (2018). Functional Assessment of Currently Employed Technology Scale (FACETS): Reliability and Validity. International Journal of Medical Science and Clinical Invention. 5. 4064-4068. 10.18535/ijmsci/v5i9.07.
- Nelson LA, Pennings JS, Sommer EC, Popescu F, Barkin SL. A 3-Item Measure of Digital Health Care Literacy: Development and Validation Study. JMIR Form Res. 2022 Apr 29;6(4):e36043. doi: 10.2196/36043. PMID: 35486413; PMCID: PMC9107049.
- van der Vaart R, Drossaert C. Development of the Digital Health Literacy Instrument: Measuring a Broad Spectrum of Health 1.0 and Health 2.0 Skills. J Med Internet Res. 2017 Jan 24;19(1):e27. doi: 10.2196/jmir.6709. PMID: 28119275; PMCID: PMC5358017.

Name: Reed, Angel

Site: Detroit Health Department - Workforce Development

Title: Enhancing Public Health Awareness: Developing Curriculum and Sparking Career Interest for Youth Engagement in Detroit

Introduction: This practicum aimed to promote early public health career exposure among Detroit middle and high school students by developing an “Introduction to Public Health” summer camp curriculum. Recognizing that early awareness of public health fundamentals improves future workforce diversity and community representation in public health careers, the practicum focused on creating an accessible and inspiring educational experience

Methods: Under supervision from the Detroit Health Department Workforce Development division, the student gathered qualitative and quantitative data to design a youth-friendly curriculum. Best practices were drawn from local and national sources, while community-based insights were obtained from staff discussions. The project integrated Public Health foundational and Public Health Practice concentration competencies, including data use, communication strategies, and the development of community-focused materials that are aligned with social determinants of health.

Outcomes: The result will include a comprehensive, five-day curriculum covering public health fundamentals, social determinants of health, environmental justice, public health careers, and youth advocacy. Each module included hands-on activities, local context, and space for reflection and discussion. The curriculum is designed for future implementation during a summer youth camp and supports efforts to build the next generation of Detroit’s public health leaders.



Enhancing Public Health Awareness: Developing Curriculum and Sparking Career Interest for Youth Engagement in Detroit

Angel Reed, CCHW, CBD, CPD, BS. Master of Public Health Candidate
Dr. Alescia M. Farr, PH.D, M.P.H. Workforce Development Director and site preceptor
City of Detroit Health Department

Introduction

This practicum fostered professional skill-building and contributed to the Detroit Health Department's mission to cultivate the next generation of public health leaders. The deliverable supports youth empowerment through education and community engagement, laying the groundwork for a sustainable pipeline into public health careers. This practicum aimed to promote early public health career exposure among Detroit middle and high school students by developing an "Introduction to Public Health" summer camp and curriculum. Recognizing that early awareness of public health fundamentals improves future workforce diversity and community representation in public health careers, the practicum focused on creating an accessible and inspiring educational experience.

Public Health Foundational & Concentration Competencies	Deliverable 1 "Introduction to Public Health" Curriculum	Deliverable 2 Program Proposal with Budget and Implementation Plan
Foundational 2 Collect data to support public health work	X	
Foundational 9 Design a population-based policy, program, project, or intervention	X	X
Foundational 18 Select communication strategies for different audiences and sectors	X	X
Concentration 1 Construct community-focused materials to address population health leadership challenges	X	X
Concentration 2 Propose public health interventions that reflect the social determinants of health	X	X

Figure 1 – Deliverables Table 1 and 2. Foundational and Concentration Competencies for practicum outcomes

Methods

The student conducted a literature review and interviewed health department staff to develop a culturally relevant curriculum. The five-day camp plan integrates Detroit-specific health challenges, hands-on learning, and career exploration. The project utilized evidence-based approaches and addressed public health foundational and public health practice concentration competencies such as data use, communication, planning, and the development of community-focused materials that are aligned with social determinants of health.

Results

- The result will include a comprehensive, five-day curriculum covering public health fundamentals, social determinants of health, environmental justice, public health careers, and youth advocacy.
- Each module included hands-on activities, local context, and space for reflection and discussion.
- The curriculum is designed for future implementation during a summer youth camp and supports efforts to build the next generation of Detroit's public health leaders.

Implementation

- Work on the implementation plan began during the first two months of the practicum, with the budget developed and refined between months 3 and 4.
- The entire process will take approximately 4–5 months, with time allotted for review, revisions, and final formatting.

References

- CDC (2023), *Essential Public Health Services*, www.cdc.gov
- CDC (2023), *Public Health Framework & Services*, www.cdc.gov
- WHO (2023), *Health Equity Report*,
- Erwin & Brownson (2022), *Principles of Public Health Practice*
- Aschengrau & Seage (2019), *Essentials of Epidemiology in Public Health*
- Last, J. (2007), *A Dictionary of Public Health*, Oxford University Press
- WHO (2023), *Social Determinants of Health Framework*,.

Conclusion

This practicum fostered professional skill-building and contributed to the Detroit Health Department's mission to cultivate the next generation of public health leaders. The deliverable supports youth empowerment through education and community engagement, laying the groundwork for a sustainable pipeline into public health careers.

Skills Building

Youth Empowerment

Community Engagement

Future Recommendation

- Involving youth earlier in the planning process through focus groups would ensure greater alignment with their interests and lived experiences.
- Creating a youth advisory board from youth who have attended the summer camp during the pilot, or active in the community.
- Creating a shared development schedule and review framework among department staff could also streamline collaboration and improve the feedback process.
- Developing pre- and post-assessments for students would help evaluate learning outcomes and support future program improvement



Name: Stilwell, Erin

Site: Henry Ford Health - Department of Family Medicine

Title: Improving Post-Acute Care Quality Through a Data-Informed Skilled Nursing Facility (SNF) Scorecard: An Exploratory Analysis to Identify Areas for Improvement and Inform Future Policy

Introduction: As skilled nursing facilities (SNFs) and subacute rehabilitation services (SARs) play a critical role in post-acute care, understanding their performance is essential for improving patient outcomes and supporting value-based care. This project conducted a landscape analysis of SNFs/SARs affiliated with the Henry Ford Medical Group (HFMG) to inform policy and enhance care coordination.

Methods: A mixed-methods approach combined quantitative analysis of Medicare Star Ratings and HFMG's SNF Scorecard data with qualitative interviews of HFMG providers and post-acute care teams. Metrics included readmission rates, length of stay, discharge outcomes, and placement trends. Geographic distribution and facility characteristics were mapped and analyzed to assess network coverage and quality indicators.

Outcomes: Findings revealed variation in SNF performance, early readmission spikes linked to provider communication gaps, and a lack of standardized quality tracking. Implementation of the SNF Scorecard enables detection of high-risk patterns, improves alignment between referral practices and facility capabilities, and informs quality improvement strategies. Interview themes emphasized the need for care navigators, enhanced discharge planning, and stronger SNF-provider collaboration.



WAYNE STATE School of Medicine

Improving Post-Acute Care Quality Through a Data-Informed Skilled Nursing Facility (SNF) Scorecard: An Exploratory Analysis to Identify Areas for Improvement and Inform Future Policy

Erin Stilwell, Master of Public Health Candidate
Denise W. Perkins, MD, PhD - Chair, Department of Family Medicine,
Director, Healthcare Equity Initiatives, Office of System Diversity Equity and Inclusion Henry Ford Health System
Henry Ford Health Department of Family Medicine, Geriatric Division

Introduction

Henry Ford Health's Geriatric Department serves older adults across Southeast Michigan through a network of skilled nursing facilities (SNFs) and subacute rehabilitation services (SARs) ¹. This project aimed to conduct a landscape analysis of the SNFs/SARs and to explore current quality metrics, identify gaps, and inform potential improvements through a combination of quantitative data analysis and healthcare professional interviews.

Landscape Analysis

A landscape analysis is a structured method of gathering, organizing, and evaluating information to understand the current state of a system. Landscape analysis of 26 SNFs/SARs and 13 Henry Ford Health System hospitals the Henry Ford Medical Group (HFMG) providers serves:

- Demonstrates geographic distribution and coverage to visualize proximity and service area coverage
- Color coded by facility overall Medicare star rating ²:
 - Green 4/5
 - Yellow 3/5
 - Orange 2/5
 - Red 1/5

HFMG Geriatric Provider SNFs/SARs



Figure 1: Landscape Analysis of HFMG provider SNFs/SARs

To view the landscape analysis, use the QR code, click lower right QR code icon, and click "Open in Safari".

Methods

Study Design

Mixed-methods approach

Quantitative

Henry Ford Post-Acute Care Services (PACS) SNF Scorecard was analyzed to assess quality indicators.

Qualitative

Semi-structured interviews were conducted with Henry Ford Medical Group (HFMG) providers and administrative staff.

Results

Quantitative Findings - Analysis of the Post-Acute Care Services (PACS) SNF Scorecard, covering seven HFMG-affiliated skilled nursing facilities, revealed significant variability in facility performance:

- 30-Day Readmission Rates varied from below 10% to over 20%, with several facilities exceeding CMS benchmarks.
- Average Length of Stay (ALOS) differed notably, impacting resource use.
- Discharge to Community Rates were lower at facilities with higher readmissions, indicating care coordination challenges.
- Medicare Star Ratings ranged from 2 to 5 stars, suggesting disparity in quality among networked SNFs.
- High-Risk Patient Placement was not consistently aligned with facility capacity or performance indicators.
- These disparities highlight opportunities to strengthen alignment between facility selection and patient needs.

Qualitative Findings - Interviews with HFMG providers and post-acute care administrators surfaced key themes:

- Lack of Standardized Metrics: Providers reported limited access to comparative SNF quality data.
- Early Readmissions: A case of elevated day 1-7 readmissions was traced to on-call coverage issues; communication improvements led to measurable declines.
- Need for Discharge Support: Participants emphasized the need for a dedicated Care Navigator to support patients and reduce burden on social workers.
- Desire for Data Transparency: Professionals expressed need for a centralized, provider-specific SNF scorecard to track facility quality and promote accountability.

Outcomes

- Wide variation was found across SNFs in readmission rates, quality ratings, and discharge performance.
- Reported early readmissions (within 7 days) were linked to on-call provider communication gaps.
- Providers reported limited tools for assessing facility quality and emphasized the need for standardized tracking.
- The SNF Scorecard was seen as a promising tool for improving transparency, referral alignment, and equity in care.

Conclusion

SNF quality and referral decisions significantly impact patient outcomes, particularly in value-based care settings. This project revealed both the potential of existing scorecard data and the urgent need for a unified system to support provider decision-making. Integrating provider insights into performance evaluation could drive quality improvement and more equitable care transitions.

Recommendations

- Develop and pilot an HFMG-specific SNF Scorecard with real-time provider input.
- Implement workflows for routine performance reviews and interdepartmental feedback.
- Invest in a Care Navigator role to support patients during discharge and reduce readmissions.
- Strengthen communication protocols between hospital teams and SNFs, especially during handoffs.

References

1. Henry Ford Health. (n.d.). Senior Services. Retrieved January 20, 2025, from <https://www.henryford.com/services/senior>
2. Centers for Medicare & Medicaid Services. (2025). Nursing Home Compare Five-Star Quality Rating System: Technical Users' Guide.

Name: Subhani, Noor

Site: Michigan Department of Health and Human Services - Bureau of Infectious Disease Prevention - Division of Immunization - Vaccine-Preventable Disease Epidemiology Section

Title: Enhancing Surveillance and Hospital Reporting of Perinatal Hepatitis B

Introduction: The Division of Immunization, Bureau of Infectious Disease Prevention, Michigan Department of Health and Human Services (MDHHS) enhances community health outcomes. This is achieved by reducing vaccine preventable diseases through immunization coverage surveillance via Michigan Care Improvement Registry (MCIR) and implementing programs to educate the public.

Methods: I conducted a literature review on the methodologies of national vaccine surveys and state level data (MCIR) to see how vaccination questions are asked to familiarize myself with the content. I then analyzed vaccination coverage rates in the African American population by creating an infographic on Canva. I developed a communication tool in a form of a survey focusing on Perinatal Hepatitis B surveillance and case management in birthing facilities and hospitals. The survey seeks input from the birthing facilities and hospital to identify opportunities for improvement.

Outcomes: I produced two deliverables. Deliverable 1 is an infographic that highlights vaccination coverage rates within the African American population by comparing data from both the Michigan Care Improvement Registry (MCIR) and national data from ChildVax View. Deliverable 2 is a survey designed to evaluate hospital and birthing facility performance in Perinatal Hepatitis B surveillance and management. It has not yet been distributed to the birthing facilities.



Enhancing Surveillance and Hospital Reporting of Perinatal Hepatitis B

B

Noor Subhani, Master of Public Health Candidate

WAYNE STATE Preceptor: Taylor Olsabeck, MS, Vaccine Preventable Disease Epidemiology Section Manager
School of Medicine Division of Immunization, Bureau of Infectious Disease Prevention, Michigan Department of Health and Human Services (MDHHS)

Introduction

“MDHHS provides services and administers programs to improve the health, safety, and prosperity of the residents of the state of Michigan.” (MDHHS Strategic Priorities, 2025).³

The Division of Immunization, Bureau of Infectious Disease Prevention, Michigan Department of Health and Human Services (MDHHS) enhances community health outcomes. This is achieved by reducing vaccine preventable diseases through immunization coverage surveillance via Michigan Care Improvement Registry (MCIR) and implementing programs to educate the public.

Methods

Surveillance Project

For the first deliverable, I was responsible for the surveillance project, which involved comparing results from national surveys to the data published by the immunization team using the Michigan Care Improvement Registry (MCIR). I developed an infographic to illustrate my findings on immunization coverage rates by comparing MCIR data to national vaccine trends from ChildVaxView, a source provided by my preceptor, specifically focusing on the African American population. The infographic was designed to highlight how vaccination coverage rates vary across racial groups due to social determinants of health.

Communication Tool

For the second deliverable, I developed a survey designed to evaluate the performance of hospitals and birthing facilities in Perinatal Hepatitis B surveillance and case management. The survey also seeks input from these facilities to identify opportunities for improvement.

Deliverables

Figure 1: Infographic

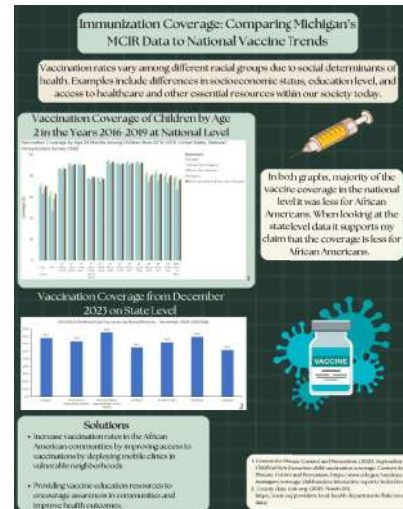


Figure 2: Survey

Deliverable 2: Communication Tool (Survey)

Section 1: Facility and Risk of Respondent

1. Facility Name:
2. County:
3. Address:
4. Main phone #:
5. Email address:
6. What is your role in the hospital?

Section 2: Program Contacts

1. Who should we contact regarding HBV/Hepatitis B?
- Name/Title:
- Phone:
- Email:

Section 3: Vaccination Information

1. Is your facility enrolled in the VQC (Universal Hepatitis B Program)?
 - a. If yes, is your program intended to learn more about the program?
2. Are you aware of the percent of infants born in your facility who receive the recommended dose of Hepatitis B?

Section 4: Perinatal Hepatitis B Case Management

1. What is your policy for when a HBV/Hepatitis B woman is present?
2. What are the steps that follow when a woman comes in with HBV/Hepatitis B?
3. Approximately how many HBV/Hepatitis B positive women deliver at your facility annually?
4. Are there any challenges in managing Perinatal Hepatitis B cases? (Please describe, including limited resources, unclear policies and procedures? If yes, please explain.)

Section 5: Reporting and North Assessment

1. Are you aware of the previous Hepatitis B Hospital Report Card?
2. If yes, did you find them beneficial?
3. How would you use the following resources for your facility? (circle 1 to 5)
 - a. Number of letters reported by Electronic Health Certificates
 - b. Number of women identified through the EHR that are HBV/Hepatitis B
 - c. Number of HBV/Hepatitis B cases identified
 - d. Number of letters sent to women's HBV/Hepatitis B cases
 - e. Number of HBV/Hepatitis B cases with women's HBV/Hepatitis B cases
 - f. Percent of infants who were administered HBV/Hepatitis B vaccine within 7 days of birth

Outcomes

Deliverable 1

Created an infographic highlighting vaccination trends within the African American population by comparing data from the Michigan Care Improvement Registry (MCIR) and national data from ChildVaxView. Both sources showed that vaccine coverage was generally lower among African Americans. This population is disproportionately affected by social determinants of health.

Deliverable 2

The survey has been completed. Next steps include developing a timeline for distributing the survey to birthing facilities and hospitals.

References

1. Centers for Disease Control and Prevention. (2020, September 28). ChildVaxView interactive child vaccination coverage. Centers for Disease Control and Prevention. <https://www.cdc.gov/vaccines/imz-managers/coverage/childvaxview/interactive-reports/index.html>
2. County data. mcir.org. (2025, March 20). <https://mcir.org/providers/local-health-departments-lhds/county-data/>
3. MDHHS Strategic Priorities FY 2023-2027. (n.d.). https://www.michigan.gov/-/media/Project/Websites/mdhhs/Folder50/Folder16/MDHHS_Strategic_Priorities_FY_2023-2027_-_Accessible.pdf?rev=e3ff9d48eadf4c4c947571d664a87288

Name: Valbrun, Shaurel

Site: Integrative Biosciences Center (IBio) - Behavioral Research Lab

Title: Examining Community Engagement and Exploring Implicit Bias

Introduction: Addressing Cardiometabolic Health Inequities by Early Prevention in the Great Lakes Region (ACHIEVE GREATER) aims to reduce cardiometabolic health disparities at an early stage for Black participants in Detroit, Michigan and Cleveland, Ohio. Achieve Greater consists of 3 Cores: Administrative, Community Engagement, and Investigator Development. The Community Engagement Core (CEC) provides training to establish community-academic partnerships and ensure meaningful community engagement across the continuum of health equity research. This project emphasized work in the CEC, specifically highlighting implicit bias.

Methods: Microsoft PowerPoint was used to create a CEC poster for the Health Equity Action Network conference (HEAN). The poster was used to inform national community members about Achieve Greater. Implicit Bias training was a 3-part virtual workshop series that spanned 6 total hours of training. The attendees of the training were community members, health workers, project staff and investigators, pilot awardees, affiliate institutions, and consortium members. Data from pre- and post-workshop assessments were collected using Qualtrics surveys. The data was extracted from Qualtrics, and the SPSS application was used to analyze data from the training to develop an abstract and case report.

Outcomes: This project is to inform individuals about CEC and to also highlight the importance of community work while exploring implicit bias. The results from the workshop highlighted significant improvements in bias awareness, self-reported intentions, and immediate behavioral changes. Meaningful community engagement efforts can reduce the impact of implicit bias on health outcomes and reduce disparities. Future work includes additional workshop and dissemination of the data for a publication.



Examining Community Engagement and Exploring Implicit Bias

Shaurel Valbrun, BA, Master of Public Health Candidate
Preceptor: Dr. Rhonda Dailey, MD Wayne State University
Integrative Biosciences Center (IBio) Behavioral Research Laboratory, Wayne State University

Introduction

Addressing Cardiometabolic Health Inequities by Early PreVENTion in the GREAT LakEs Region (ACHIEVE GREATER) aims to reduce cardiometabolic health disparities at an early stage for Black Participants in Detroit, Michigan and Cleveland, Ohio.

- ACHIEVE GREATER consists of 3 Cores:
 - Administrative
 - Community Engagement
 - Investigator Development
- The Community Engagement Core (CEC) provides training:
 - To establish community-academic partnerships
 - Ensure meaningful community engagement across the continuum of health equity research.

This project emphasized work in the CEC, specifically highlighting the community workers and implicit bias.

Methods

Data Collection

- Reviewed ACHIEVE GREATER mission and projects for Health Equity Action Network conference (HEAN) poster.
- Implicit Bias training was a 3-part virtual workshop series that spanned 6 total hours.
- The attendees of the training were community members (CM), academic researchers: health workers, project staff and investigators) (AR), pilot awardees, affiliate institutions, consortium members.
- Data from Pre and Post assessments were collected using Qualtrics surveys.

Data Analysis

- Microsoft PowerPoint was used to create CEC poster for the HEAN conference.
- The data was extracted from Qualtrics.
- The SPSS application was used to analyze the data using T-test and descriptive statistics.

Deliverable 1 - Health Equity Action Network Conference Posters



Figure 1

Figure 2

Deliverable 2 - Implicit Bias Data

- There were 46 attendees and 44 (95%) completed an assessment (62.2% AR and 15.6% CM).
- Most of the participants identified as female (71.7%), white (45.7%) and resided in Michigan (67.4%).
- The most common reason for workshop attendance for all participants was professional and personal development (73.9%).
- We found statistically significant gain in knowledge on the 16 topics ($p < .001$).
- Qualitative feedback revealed high levels of workshop satisfaction; self-reflection on bias and privilege; learning new definitions; and group learning and sharing.
- Nearly all respondents reported plans to apply their acquired skills, and some respondents used their skills between workshops.
- Recommendations included longer workshops and mandatory attendance for ARs and CMs.



Figure 3

Conclusion

Figure 1 and Figure 2

- The poster was used to inform national community members about ACHIEVE GREATER at the Health Equity Action Network Conference.

Figure 3

- The 3-Part Implicit Bias Workshop series engaged members from diverse backgrounds, fostering co-learning, knowledge sharing, and skill-building.
- The workshops resulted in significant improvements in bias awareness, self-reported intentions, and immediate behavioral changes.
- Our approach is unique in incorporating both ARs and CMs, whereas most training programs focus exclusively on ARs.
- Additionally, co-moderation by CMs enhanced their capacity to influence change within academic institutions.

Outcome

- This project was to inform individuals about CEC and highlight the importance of community work through exploring implicit bias.
- The results from the workshop highlighted significant improvements in bias awareness, self-reported intentions, and immediate behavioral changes.
- Meaningful community engagement efforts can reduce the impact of implicit bias on health outcomes and reduce disparities.

Next Steps

Future work includes additional workshops and dissemination of the data for a publication.

Acknowledgements

I would like to thank Dr. Rhonda Dailey and the entire Achieve Greater staff for their support during my practicum.

Name: Vora, Jenna

Site: Corktown Health Center

Title: Improving Health Equity through Human Immunodeficiency Virus (HIV) Care, Prevention, and Treatment

Introduction: Corktown Health Center in Detroit, Michigan is the state's first medical center to focus on health disparities in the community specifically in the Lesbian, Gay, Bisexual, Transgender, and Queer plus (LGBTQ+) community. The facility works with healthcare professionals and medical staff to provide quality of care for prevention of HIV. Their focus is to advance health and well-being for all and achieve health equity for all.

Methods: Corktown Health Center's behavioral health services department is involved in evidence informed intervention called I2Tec. It uses a video-based counseling series and two-way text messaging platform to help people living with HIV to address barriers linked to mental health. I created an infographic bar chart to look at changes in viral load suppression levels from clients in the I2Tec project. I created a flyer about the I2Tec project that will help raise awareness about the program and improve treatment adherence for people living with HIV.

Outcomes: Disseminating the infographic and flyer will help inform the public about the value and effectiveness of HIV prevention and the I2Tec program backed by evidence-based information. The health resources will continue to support and help patients gain access to behavioral health services.



Improving Health Equity in Human Immunodeficiency Care, Prevention, and Treatment

Jenna Vora, BS, Master of Public Health Candidate

Preceptor: Jennifer Harvey, LMSW Behavioral Health Manager, Corktown Health Center

Introduction

Corktown Health Center is Michigan's first medical center that focuses on health disparities in the Lesbian, Gay, Bisexual, Transgender and Queer community. Healthcare professionals and medical staff at the center offer quality care especially to people living with human immunodeficiency virus (HIV) in the city of Detroit [1]. Their focus is to improve health equity for all. Corktown Health Center is in Detroit, Michigan.

One behavioral health service department project is called I2TEC. It is an evidence-based informed intervention on telehealth and a two-way texting platform focusing on medication adherence.



Methods

My first set of activities at Corktown Health Center was learning the basic foundational information of Human Immunodeficiency Virus (HIV) through training modules. My next steps were reading about the background information of the I2TEC project which included the purpose and overview of it. Additionally, I read articles and current trends in data collection about the Human Immunodeficiency Virus (HIV).

Work Activities included:

- Read through data statistics and resources on the HIV epidemic, Human Immunodeficiency Virus (HIV)
- Analyzed Ryan White HIV client data.
- Created a Viral Load Change Chart using Excel.
- Created an Informational Flyer about the I2TEC Project using Canva.

Deliverables

Figure 1- Infographic representing the total number of Ryan White clients and their changes in viral load and the importance of viral load suppression for community members, health care providers, and stakeholders.



Figure 2- An informational flyer to support people living with HIV, improve medication adherence and be presented at an upcoming conference.



Results

- Created a pie chart that shows 70% of Ryan White clients in the I2TEC program experienced a decrease in viral load, 21% remained the same, and 9% had an increase. The infographic highlights the importance of U=U (Undetectable = Untransmissible) in reducing stigma and promoting public health. It also reinforces the importance of antiretroviral therapy (ART) in achieving viral load suppression.
- The flyer describes I2TEC as a telehealth and text-messaging counseling program aimed at improving adherence to care. It includes positive feedback from past participants about the openness, support, and convenience of the program. It lists key benefits such as better medication adherence, supportive counseling, and improved health outcomes, including viral load suppression.

Conclusion

The data analysis from the infographic helps to paint a picture which shows that 70% of participants experienced a decrease in viral load, reinforcing the success of the program in supporting viral suppression. The U=U message (Undetectable = Untransmissible) is a crucial public health principle that helps reduce stigma and improve medication adherence.

Telehealth and two-way text messaging provide an accessible and judgment-free space for clients to discuss their health, receive counseling, and improve medication adherence. Participant feedback highlights the program's positive impact on engagement and motivation in managing HIV. This highlights the importance of scaling such programs to further enhance HIV care and reduce stigma.

Reference

1. Corktown Health. Home - Corktown Health. Corktown Health. <https://corktownhealth.org/>. Published September 15, 2023.

Name: Wallace, Aimee

Site: J.O.B. Lactation Foundation

Title: Identifying barriers to breastfeeding services to address health disparities for families in Southeastern Michigan

Introduction: J.O.B. Lactation Foundation provides education and services for families to support breastfeeding. Their mission is two-fold: empowering families and advocating for health equity. The purpose of this practicum is to identify the most common barriers faced by breastfeeding individuals in Southeastern Michigan and to propose strategies that overcome these barriers, which may include the use of virtual reality (VR).

Methods: This project utilized both quantitative data found in the academic literature and qualitative data collected through interviews with breastfeeding clients of the J.O.B. Lactation Foundation. The academic literature review investigated the most common barriers to breastfeeding experienced by underserved communities throughout the United States, as well as the use of VR in breastfeeding education. Client interviews were conducted via phone calls, responses were recorded, and a thematic analysis was done to identify common themes to breastfeeding barriers.

Outcomes: 6 clients agreed to participate in the interview. The responses revealed the most common barriers of breastfeeding were lack of knowledge and support, discouragement, and the ease of formula. 15 research articles were used in the academic literature review, with 3 of articles investigating the use of VR for postpartum mothers. The research highlighted additional barriers, including workplace policies, insufficient milk supply, and cultural norms. These findings were compiled into a needs assessment and recommendations were suggested to overcome these barriers.



WAYNE STATE
School of Medicine

Identifying Barriers to Breastfeeding Services to Address Health Disparities for Families in Southeastern Michigan

Aimee Wallace, Master of Public Health Candidate
Micia Eddins, MPH, IBCLC, CD, CHW, CLS
J.O.B. Lactation Foundation

Introduction

J.O.B. Lactation Foundation provides education and services for families to support breastfeeding. Their mission is two-fold: empowering families and advocating for health equity. The purpose of this practicum is to identify the most common barriers faced by breastfeeding individuals in Southeastern Michigan and to propose strategies that overcome these barriers, which may include the use of virtual reality.

Methods

This project utilized both quantitative data found in the academic literature and qualitative data collected through interviews with breastfeeding clients of the J.O.B. Lactation Foundation. The academic literature review investigated the most common barriers to breastfeeding experienced by underserved communities throughout the United States, as well as the use of virtual reality in breastfeeding education. 15 research articles were included in the academic literature review, with 3 of the articles investigating the use of virtual reality for postpartum mothers. For the client interviews, 6 clients agreed to participate. Interviews were conducted via phone calls, responses were recorded, and a thematic analysis was performed to identify common themes to breastfeeding barriers for mothers residing in Detroit, Michigan.



Outcomes

The client interviews revealed the most common barriers of breastfeeding they experienced were

- Lack of knowledge and support
- Discouragement
- The ease of formula

The literature review highlighted these additional barriers

- Workplace policies¹
- Insufficient milk supply²
- Cultural norms²

These findings were compiled into a needs assessment and recommendations were suggested to overcome these barriers.



Recommendations

1. Increase the accessibility of breastfeeding education for both breastfeeding mothers and the families of these mothers
2. Promote policy that permits mothers to breastfeed in the workplace
3. Strengthen the breastfeeding community through events where breastfeeding mothers can be educated, resources can be advertised, and connections can be formed to promote breastfeeding as the norm
4. Investigate the use of virtual reality in "supporting mother-infant relationships and infant feeding"³

Conclusion

The breastfeeding barriers that Southeastern Michigan mothers are facing match those in other underserved United States communities, especially for Black mothers in urban cities.

Most common barriers: These barriers were due to lack of breastfeeding education, lack of familial and community support, mental health, and the difficulty of breastfeeding⁴.

Overcoming the barriers: However, these barriers can be tackled through public health interventions that prioritize education, provide support, advocate for breastfeeding rights in the workplace, and utilize virtual reality.

Virtual reality: Although little research has been done on the use of virtual reality in breastfeeding education, future studies should dedicate time to investigate how it could benefit breastfeeding mothers. Virtual reality could be especially helpful for Black women as it can be customized to individual needs specific to this community and may be more accessible due to its in-home use.

References

1. Butler, M., Allen, J. A., Hoskins-Wroten, J., Sanders-Bey, T., Venegas, R. N., Webb, I., & Ragland, K. (2021). Structural Racism and Barriers to Breastfeeding on Chicagoland's South Side. *Breastfeeding medicine : the official journal of the Academy of Breastfeeding Medicine*, 16(2), 112–115.
2. Buxbaum, S. G., Arigbede, O., Mathis, A., Close, F., & Darling-Reed, S. F. (2023). Breastfeeding among Hispanic and Black Women: Barriers and Support. *Journal of biomedical research & environmental sciences*, 4(8), 1268–1273.
3. Blanc, J., Scaramutti, C., Carrasco, M., Dimanche, S., Hollimon, L., Moore, J., Moise, R., Gabbay, V., & Seixas, A. (2024). I am Lifted Above the World: utilizing VR for stress reduction among perinatal women of color. *Frontiers in psychiatry*, 15, 1377978.
4. Pezley, L., Cares, K., Duffecy, J., Koenig, M. D., Maki, P., Odoms-Young, A., Clark Withington, M. H., Lima Oliveira, M., Loiacono, B., Prough, J., Tussing-Humphreys, L., & Buscemi, J. (2022). Efficacy of behavioral interventions to improve maternal mental health and breastfeeding outcomes: a systematic review. *International breastfeeding journal*, 17(1), 67.

Name: Ware, Sharamie

Site: Michigan site of the Midwest AIDS Education and Training Center (MATEC)

Title: Human Immunodeficiency Virus (HIV) is Not a Crime: Criminalization laws Analysis: A Review of HIV laws, law reform, and the impact on the lives of People Living with HIV (PLWH)

Introduction: Human Immunodeficiency Virus has long been a significant public health concern. The criminalization of HIV transmission has emerged as a complex issue. This literature offers a comparative analysis of the criminalization of HIV transmission, focusing on national trends and the case of Michigan. Many states have laws that criminalize the non-disclosure of HIV status or the intentional transmission of the virus. Proponents argue that these laws deter HIV transmission and promote public health safety; contenders that they disproportionately affect marginalized groups. In Michigan, the criminalization of HIV is particularly controversial due to the state's legal framework that includes felony charges for the potential transmission of HIV. This document explores how Michigan's approach to HIV criminalization compares with national practices. Highlights include the importance of shifting from punitive approaches to more health-focused interventions.

Methods: This comparative analysis employed qualitative and quantitative peer reviewed research to evaluate similarities and differences among various states in comparison to the state of Michigan. Data was gathered through document review. Criteria for comparison were reviewed in order to construct a literary review to contribute to scholarly overview of that laws that impact the lives of persons living with HIV. Maps were used to visualize the states that have differentiating laws. Findings provide evidence-based insights to guide future decisions and policy development.

Outcomes: HIV laws have not been found to deter the transmission of HIV; therefore, HIV laws should be changed to reflect language that includes evidence-based approaches that mitigate the weaponizing of HIV status.

Human Immunodeficiency Virus (HIV) is Not a Crime, Criminalization Laws Analysis: A Review of HIV laws, law reform, and the impact on the lives of People Living with HIV (PLWH)

Sharamie Ware, Master of Public Health Candidate
Amanda Allmacher, DNP, RN, NP-BC, AAHIVE, Associate Director
Midwest AIDS Training + Education Center

Introduction

Introduction:

Human Immunodeficiency Virus (HIV) has long been a significant public health concern, but the criminalization of HIV transmission has emerged as a complex issue, particularly within the United States. This literature offers a comparative analysis of the criminalization of HIV transmission, focusing on national trends and the specific case of Michigan. Many states have implemented laws that criminalize the non-disclosure of HIV status or the intentional transmission of the virus, sparking debate regarding their effectiveness, ethical implications, and potential harm. Proponents argue that these laws deter HIV transmission and promote public health safety, while critics contend that they disproportionately affect marginalized groups, including people of color, LGBTQ+ individuals, and low-income communities. Moreover, such laws may undermine public health efforts by discouraging prevention, status disclosure, and treatment. In Michigan, the criminalization of HIV is particularly controversial due to the state's legal framework that includes felony charges for the potential transmission of HIV, suggesting that penalties significantly reduce the spread of the virus. This paper explores how Michigan's approach to HIV criminalization compares with national practices, examining the broader implications for public health, human rights, and social justice. The study highlights the importance of shifting from punitive approaches to more effective, health-focused interventions that prioritize education, prevention, and care for individuals living with HIV.

The state of Michigan made a total of five amendments to its HIV laws in 2019¹. The following are now associated with Michigan law²:

- Reform which removed specific types of physical contact not likely to transmit HIV.
- Anal or vaginal sex without first disclosing status with a "specific intent" to transmit is a felony (up to 4 years). Transmission is not required for prosecution.
- Reckless exposure: sexual activity prior to disclosure resulting in transmission but without intent to transmit is a felony (up to 4 years).
- Reckless exposure without transmission is a misdemeanor.
- PLWH who can show that they have been virally suppressed for at least six months and are following their physician's treatment plan may use that as a defense to prove that they did not act with reckless disregard. Does not apply to acting with intent to transmit provisions of the new law.

There are studies that suggest decriminalizing HIV has little to no impact on new transmissions³, there is not yet enough data to associate whether reform to the laws that criminalize HIV have an impact on new diagnoses. However, there is support that demonstrates how criminalizing HIV increases harm and perpetuates stigma towards PLWH⁴.



Methods

Research and data collection using peer-reviewed sources and materials. This comparative analysis employed both qualitative and quantitative peer reviewed research to evaluate similarities and differences among various states in comparison to the state of Michigan. Data was gathered through document review. Criteria for comparison were reviewed in order to construct a literary review to contribute to scholarly overview of that laws that impact the lives of persons living with Human Immunodeficiency Virus (HIV). Maps were used to visualize the states that have differentiating laws. The methodology was used to ensure objectivity, reliability, and relevance, allowing for an in-depth understanding of key differences and shared features. Findings provide evidence-based insights to guide future decisions and policy development.

Results

Progress in Law Reform

Several states have begun aligning laws with scientific and public health perspectives:

- **Michigan, Louisiana, North Carolina, and Iowa** have:
 - Integrated **scientific evidence** (e.g., undetectable = untransmittable)
 - Emphasized **harm reduction** and public health strategies

Opportunities for Improvement

- **Retroactive Case Reviews:**
 - Reevaluate cases where no transmission occurred
 - Consider sentence reductions for non-disclosure without transmission
- **Health Equity Investments:**
 - Address **social determinants of health**
 - Align legal frameworks with **current scientific understanding**
 - Focus especially on states in the **Southern U.S.**, where disparities are greatest

Education as Prevention

- Increase awareness of:
 - HIV-related laws
 - Sexual health and prevention strategies
- Target audiences:
 - Individuals newly diagnosed with HIV
 - Populations engaged in higher-risk activities



Figure 2 – States with specific and enhanced HIV laws

Conclusion

Reforming HIV criminalization laws through a science-based, public health-oriented, and equity-focused lens can:

Reduce stigma

Improve health outcomes

Support harm reduction efforts across communities

Figure 1:
https://www.hivlawandpolicy.org/sites/default/files/Timeline%20of%20State%20Reforms%20and%20Repeals%20of%20HIV%20Criminal%20Laws%20062022_0.pdf

Figure 2:
<https://aidsvu.org/news-updates/hiv-is-not-a-crime-awareness-day-toolkit-2024/>

References

1. . Michigan HIV & STD Law Update Summary. (n.d.-e). https://www.michigan.gov/-/media/Project/Websites/mdhhs/Folder3/Folder70/Folder170/Folder1/Folder270/Michigan_HIV_STD_Law_Updates_FAQ.pdf?rev=3a6d3782fa34b9483799a8836e7f490
2. Timeline of State Reforms and Repeals of HIV Criminal Laws: States with most significant changes to their HIV laws. (n.d.-h). https://imgaleg.maryland.gov/cmtc_testimony/2024/jud1LxStUkC3RmVY_s63mASFAzTqK5S5BU.pdf
3. *AIDSVu* launches 2022 data and interactive maps, places emphasis on Health Equity's role in the HIV epidemic. *AIDSVu*. (2024, September 11). <https://aidsvu.org/news-updates/aidsvu-launches-2022-data-and-interactive-maps-places-emphasis-on-health-equitys-role-in-the-hiv-epidemic/#:text=In%202022%2C%2037%2C966%20people%20were,ever%20been%20tested%20for%20HIV.>
4. Pappas, S. (n.d.). *HIV laws that appear to do more harm than good: A movement is afoot to change HIV legislation.* Monitor on psychology. <https://www.apa.org/monitor/2018/10/ce-corner>

Thank you to the Midwest AIDS Training + Education Center for the opportunity to explore information that may contribute to the continuous improvement of the lives of people living with HIV and the quality of life for the communities we serve.

Name: Warmbrunn, Isabella

Site: Henry Ford Health - Department of Family Medicine

Title: Historical Community Oriented Primary Care Evaluation Report (2004-2024)

Introduction: The Community Oriented Primary Care (COPC) module in the Henry Ford Health System was implemented in 1993 to provide family medicine residents with epidemiological experience in population health research. The goal was for residents to learn how to define health needs within the local community and create a scholarly project using data from the Henry Ford electronic health record. The purpose of this survey evaluation is to assess the impact of COPC on research productivity and fulfilling American Board of Family Medicine requirements.

Methods: I had two main practicum projects. The first was independently conducting a historical literature review of 2004-2024 COPC cohorts and evaluating how many scholarly works (academic publications or presentations) were published after completing their COPC module. My second practicum project was the creation, distribution of a historical COPC cohort survey to 2004-2024 COPC residents with scholarly work published after their COPC module.

Outcomes: 41 of the 152 physicians who completed the COPC module in 2004-2024 matched the inclusion criteria—completing scholarly work (academic publications or presentations) after their COPC module was completed. 10 surveys were completed with a majority (n=7) responses having completed residency in the past 1-5 years. 80% of respondents agreed the COPC rotation helped fulfill American Board of Family Medicine requirements. 60% of respondents agreed the COPC rotation increased knowledge of population health and future clinical and quality improvement practices. On the free response question, most respondents indicated the COPC module was beneficial in engaging in clinical research and enjoyed working with co-residents.



WAYNE STATE
School of Medicine

HENRY FORD HEALTH

Historical Community Oriented Primary Care Evaluation Report (2004-2024)

Isabella Warmbrunn, MS, Master of Public Health Candidate

Preceptor: Katarzyna Budzynska, MD, MSc, Director of Research for Department of Family Medicine, Assistant Program Director for Scholarly Activity, Henry Ford Health – Department of Family Medicine

Introduction

The Community Oriented Primary Care (COPC) module in the Henry Ford Health System was implemented in 1993 to provide family medicine residents with epidemiological experience in population health research. Additionally, Population health and epidemiology play a critical role in modern medical practice, influencing patient outcomes at both individual and community levels (1-4). The goal of the COPC module is for residents to learn how to define health needs within the local community and create a scholarly project using data from the Henry Ford electronic health record.

This module has never been evaluated for continued scholarly work (e.g., academic publications or presentations) engagement after residency completion. The purpose of conducting a literature review of past COPC residents to understand the topics past COPC residents published on and frequency of publication after residency. The creation and distributing of survey to past COPC cohort members engaged in continued scholarly activity will assess the impact of COPC on research productivity and impact of COPC in fulfilling the American Board of

Methods

Study Design: Retrospective cohort design of residents completing COPC rotation between 2004 and 2024, incorporating a literature review and bibliometric analysis to understand trends in cohort.

Inclusion Criteria: Physicians who have completed the COPC rotation and engaged in scholarly work (e.g., academic publications or presentations) between 2004 and 2024.

Data Collection: An electronic survey was developed using Microsoft Forms and emailed to physicians matching inclusion criteria.

I had two main practicum projects.

- Literature Review:** independently conducting a historical literature review of 2004-2024 COPC cohorts and evaluating how many scholarly works (academic publications or presentations) were published after completing their COPC module. This was done in an Excel file and including bibliometric analysis and quantitative findings (number of scholarly works) since completion of residency.
- Data Collection:** The creation, distribution, and analysis of a historical COPC cohort survey. The survey was created in Microsoft Forms and sent to 2004-2024 COPC residents with scholarly work published after their COPC module. Questions were designed to assess the impact of the COPC module in advancing goals of research engagement and success in achieving Board of Family Medicine requirements.

Survey

1. By clicking "Agree" below, you electronically consent to participate in this Historical Community Oriented Primary Care Evaluation Survey conducted by Henry Ford. By clicking "Agree" below you are signifying that you have read and understood the provided information regarding the study's purpose, procedures, potential risks and benefits, confidentiality measures, and your right to withdraw at any time without penalty. You acknowledge that your anonymous data will be collected and analyzed in accordance with ethical research guidelines. If you have any questions, please contact the Study Coordinator (Isabella Warmbrunn, iwarmbr1@hfhs.org) or Principal Investigator Dr. Katarzyna Budzynska (kbudzyn1@hfhs.org) with any further questions. *

☐ Agree

☐ Disagree

2. When did you complete your Family Medicine Henry Ford residency?

☐ In the past 0-5 years

☐ In the past 5-10 years

☐ In the past 10-20 years

☐ over 20 years ago

3. Participating in the COPC rotation helped to fulfill American Board of Family Medicine requirements (e.g., performance improvement activity) *

Strongly Disagree Disagree Neither Agree nor disagree Agree Strongly Agree

☐ ☐ ☐ ☐ ☐

4. Participating in the COPC rotation increased my knowledge and awareness of population health *

Strongly Disagree Disagree Neither Agree nor disagree Agree Strongly Agree

☐ ☐ ☐ ☐ ☐

5. The COPC rotation helped me in my future clinical and Quality Improvement (QI) practice *

Strongly Disagree Disagree Neither Agree nor disagree Agree Strongly Agree

☐ ☐ ☐ ☐ ☐

6. The COPC rotation helped me incorporate quality improvement initiatives to my practice *

Strongly Disagree Disagree Neither Agree nor disagree Agree Strongly Agree

☐ ☐ ☐ ☐ ☐

Deliverable 2 - Historical COPC 2025 cohort evaluation survey questions 1-6

Survey (cont.)

7. The COPC rotation helped me incorporate research projects to my practice *

Strongly Disagree Disagree Neither Agree nor disagree Agree Strongly Agree

☐ ☐ ☐ ☐ ☐

8. Do you engage in ongoing scholarly activities (e.g., academic publications and posters) ?

☐ Yes

☐ No

9. Any other information you would like to share with us about COPC rotation? Do not include protected health information including specific patient scenarios.

(If no additional feedback, enter None) *

Deliverable 2 - Historical COPC 2025 cohort evaluation survey questions 7-9

Results

- 152 Family Medicine residents completed the COPC module from 2004 to 2024, 41 met inclusion criteria and were emailed the Microsoft Form survey
- Literature Review:** 27 physicians completed only one scholarly work post COPC, 7 had two scholarly works post COPC, 2 had four scholarly works post COPC and four has 5+ scholarly works post COPC
- Survey:** 10 responses
- A majority (n=7) respondents completed residency in the past 1-5 years.
- 80% of respondents agreed the COPC rotation helped fulfill American Board of Family Medicine requirements.
- 60% of respondents agreed the COPC rotation increased knowledge of population health and future clinical and quality improvement practices.
- On the free response question, most respondents indicated the COPC module was beneficial in engaging in clinical research and enjoyed working with co-residents

Conclusion

Based on the responses from this survey, the COPC module encourages continued research engagement after residency and equips physicians the necessary skills to continue to engage in quality improvement practice and fulfilling American Board of Family Medicine requirements. Additionally, the module seems to reinforce teamwork, engagement in research and desire to engage in future scholarly activities.

References

- Lochner J, Lankton R, Rindfleisch K, Arndt B, Edgoose J. Transforming a Family Medicine Residency Into a Community-Oriented Learning Environment. Fam Med. 2018;50(7):518-525. <https://doi.org/10.22454/FamMed.2018.118276>.
- American Academy of Family Physicians (AAFP). Integration of Primary Care and Public Health (Position Paper). Access August 30, 2024. <https://www.aafp.org/about/policies/all/integration-primary-care.html>
- O'Connor PJ. Community-oriented primary care in a brave new world. Arch Fam Med. 1994;3(6):493-494.
- Longlett SK, Kruse JE, Wesley RM. Community-oriented primary care: critical assessment and implications for resident education. J Am Board Fam Pract. 2001;14(2):141-147.

Name: Woods, Natalie

Site: Birth Detroit

Title: Improving On-site Medication Access for Detroit Mothers Seen at Birth Detroit, who Test Positive for Sexually Transmitted Infections Through HRSA 340B Designation

Introduction: Birth Detroit is a community based non-profit birthing center that provides prenatal and postpartum care by midwives to Detroit families. They focus on addressing disparities in maternal and infant health outcomes. Birth Detroit's mission is rooted in empowering women and families, with services to ensure safe, dignified, and culturally responsive care. Despite their commitment to addressing healthcare inequities, they currently lack medication resources to provide treatment on-site for sexual transmitted infections (STI) to positive mothers. Currently, Detroit has the highest STI rates in the Nation with Congenital Syphilis rates on the rise. "Since 2019, congenital syphilis births in Michigan have increased more than 200%," making this agenda a top priority in perinatal care (Michigan Department of Health and Human Services, 2025).

Methods: Using a retrospective chart review of obstetrical patients seen at Birth Detroit during 2023 and 2024, data was collected from patients who tested positive for vaginal infections during this two-year period. The data was then categorized into five groups: No Infection, Chlamydia, Bacterial Vaginosis, Vaginal Yeast Infections, and Trichomonas. I used descriptive statistics to determine the frequency and percentages of vaginal infections in these patients to gain a better understanding of the prevalence of STIs at the birth center.

Outcomes: I was able to create a plan where the birth center could partner with the local health department that would allow the birth center to become a contracted service provider under their 340B-covered entity status. This would grant the birth center access to 340B-priced medications through contracted pharmacies without having to apply as an independent 340b site.



WAYNE STATE School of Medicine

Improving On-site Medication Access for Detroit Mothers Seen at Birth Detroit Who Test Positive for Sexually Transmitted Infections Through HRSA 340B Designation

Natalie Woods MSN, CNM, Master of Public Health Candidate
Preceptor: Char'ly Snow, MSN, CNM, Clinical Health Officer Birth Detroit

Introduction

Introduction: Birth Detroit is a community based non-profit birthing center that provides “prenatal and postpartum care by midwives (Birth Detroit, 2021.)” to Detroit families. They focus on addressing disparities in maternal and infant health outcomes. Birth Detroit’s mission is rooted in empowering women and families, with services to ensure safe, dignified, and culturally responsive care. Despite their commitment to addressing healthcare inequities, they currently lack medication resources to provide treatment on-site for sexual transmitted infections (STI) to positive mothers. Currently, Detroit has the highest STI rates in the Nation with Congenital Syphilis rates on the rise. “Since 2019, congenital syphilis births in Michigan have increased more than 200% (Michigan Department of Health and Human Services, 2025.)”, making this agenda a top priority in perinatal care.

Methods

Methods: Using a retrospective chart review of obstetrical patients seen at Birth Detroit during 2023 and 2024, data was collected from patients who tested positive for vaginal infections during this two-year period. The data was then categorized into five groups: No Infection, Chlamydia, Bacterial Vaginosis, Vaginal Yeast Infections, and Trichomonas. I used descriptive statistics to determine the frequency and percentages of vaginal infections in these patients to gain a better understanding of the prevalence of STIs at the birth center.

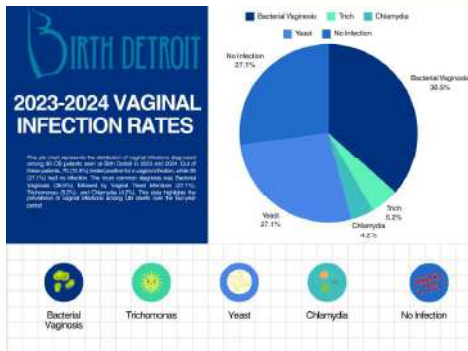


Figure 1 – Vaginal Infection Rates 2023-2024

Deliverables

- Complete and Submit the HRSA 340B Contracted Service Provider Memorandum of Understanding (MOU) with the Local Health Department (Figure 2)
- Compile a Needs Assessment Report Outlining Current Challenges, Gaps, and Areas for Improvement (Figure 1)

Results



Figure 2 – MOU between Birth Detroit and the Detroit Health Department

Implementation Plan



Figure 3 Implementation Plan

Conclusion

In conclusion I was able to create a plan where the birth center could partner with the local Health Department that would allow the birth center to become a contracted service provider under their 340B-covered entity status. This would grant the birth center access to 340B-priced medications through contracted pharmacies without having to apply as an independent 340b site.

References

- Birth Detroit. (n.d.). *About us*. Retrieved January 26, 2025, from <https://www.birtheetroit.com/about>
- Detroit Health Department & Birth Detroit. (2025). *Memorandum of Understanding between the Detroit Health Department and Birth Detroit*. Detroit Health Department.
- Michigan Department of Health and Human Services. (n.d.). *Syphilis*. Michigan.gov. Retrieved January 26, 2025, from <https://www.michigan.gov/mdhhs/keep-mi-health/chronicdiseases/hivsti/syphilis#:~:text=Since%202019%2C%20congenital%20syphilis%20births,45%20and%20in%20heterosexual%20people>

